

ENGLISH FOR PATIENT-FACING CLINIC TEAMS

Medical Assisting English

Listen carefully. Clarify the details.



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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For medical assistants and outpatient clinic staff supporting patients, clinicians, and care coordination.

2026 edition

Eight scenarios. Eight skills.

Eight clinic cases: visit expectations, patient reports, medication lists, specimen records, referrals, instruction questions, clinical calls, and appointment corrections.

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Accurate details. Respectful conversations.

Medical assistants move between patient language, clinical terminology, and administrative systems. Clear communication preserves what the patient said, distinguishes a recorded event from a judgment, and makes the next responsible person visible.

Attribute a report

Keep the source attached to information. A patient report, an observation, a measurement, and a clinician assessment have different meanings. Do not turn one into another to make a note sound more authoritative.

"Pat reports tiredness since Monday; I have not assessed its cause."

Name the unresolved detail

A useful clarification states exactly what differs. Read the appointment type, medicine-list entry, event time, or message back before proposing a correction. A discrepancy does not prove which source is wrong.

"The local list still includes it; the patient reports an outside instruction."

Give a concrete route

A role boundary should lead to the right person or process. Preserve the question and explain the immediate next step. Do not leave a clinical concern waiting for a routine administrative update.

"Nurse Dana is available through our clinical-call route now."

Separate progress from completion

Received does not mean booked, and an attempted call does not mean the patient received the correction. Name the actual stage, unfinished work, and owner so a colleague can continue accurately.

"The referral is under review; Friday is a status check, not an appointment."

PROFESSIONAL BOUNDARIES

All patients, records, times, and cases are fictional. This book teaches workplace English, not clinical assessment, triage, prescribing, specimen procedures, legal advice, or professional certification. Duties depend on local law, training, delegation, supervision, and clinic policy. Follow actual clinical and emergency procedures; these dialogues do not establish that any symptom can safely wait. US sources provide context, not universal rules. Use actual dates and required identifiers in real records.

The schedule and the expectation

A Read the situation

Clarify a mismatch between a scheduled visit and a patient concern while respecting the preferred name.

The schedule lists an annual review for patient Alexandra, whose verified record has already been matched through the clinic process. The patient prefers Alex and expected to discuss a recent concern. Assistant Sam cannot decide whether the appointment type is suitable. Nurse Dana is available to clarify the visit plan. No decision about the concern, billing, urgency, or an additional appointment has been supplied. Sam must acknowledge the expectation, preserve accurate identification, use Alex in conversation, and ask Dana to clarify the clinical plan.

PEOPLE / Alex: Patient | Sam: Medical assistant

RESPECT THE PERSON AND VERIFY THE RECORD

A preferred name and a verified record name can coexist. Use the requested form of address while following the actual identification process. A mismatch in visit expectations deserves clarification rather than blame for booking or an automatic refusal to discuss the concern.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the schedule show?

- | | |
|--|---|
| A An annual review | B A confirmed procedure for the recent concern |
| C An emergency assessment already completed | D A canceled visit |

2. What should Sam do with the name preference?

- | | |
|--|---|
| A Open another record under Alex to avoid the mismatch. | B Replace verified identifiers without following the correction process. |
| C Use Alexandra in conversation until the appointment type changes. | D Use Alex while preserving the verified record match. |

3. Who can clarify the visit plan?

- | | |
|----------------------------------|--|
| A Another waiting patient | B A billing code alone |
| C Nurse Dana | D Sam making an independent clinical decision |

[Answers and explanations: page 85.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

check-in

Arrival process confirming a patient and the scheduled visit.

IN USE complete check-in

appointment type

Category describing the scheduled purpose or format of a visit.

IN USE verify the appointment type

annual review

Periodic review visit recorded as occurring yearly.

IN USE attend an annual review

presenting concern

Issue the patient brings to the current visit.

IN USE record the presenting concern

chief complaint

Clinical-record term for the main reason for seeking care.

IN USE clarify the chief complaint

visit agenda

Topics planned for discussion during the encounter.

IN USE clarify the visit agenda

preferred name

Name a person asks others to use when addressing them.

IN USE use the preferred name

recorded name

Name shown in the relevant patient record.

IN USE verify the recorded name

patient identifier

Information used to match a person with the correct record.

IN USE confirm patient identifiers

demographic details

Administrative identity and contact information in a record.

IN USE verify demographic details

registration

Creation or updating of required administrative patient information.

IN USE complete registration

encounter

A particular interaction or visit involving healthcare services.

IN USE document the encounter

Precision check

An appointment label describes a scheduled category, not a clinical judgment about every concern raised that day. Do not promise that the concern will be resolved, reject it automatically, or invent a billing outcome from the schedule alone.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

appointment slot

Reserved time in a schedule.

IN USE confirm the appointment slot

scheduled purpose

Reason for a visit as recorded in the booking.

IN USE state the scheduled purpose

patient expectation

What a patient believes will happen during a service or visit.

IN USE acknowledge the patient expectation

clinical suitability

Whether a plan or appointment is appropriate for the clinical need.

IN USE refer a suitability question

scope of practice

Activities permitted within a role under applicable requirements.

IN USE work within scope of practice

delegation

Assignment of an activity under the applicable professional rules.

IN USE clarify the delegation

supervision

Required oversight of work by an appropriate professional.

IN USE confirm supervision arrangements

clinical handoff

Transfer of relevant information to a receiving clinical professional.

IN USE complete a clinical handoff

visit plan

Intended approach to the current appointment.

IN USE clarify the visit plan

billing category

Classification used for charging or reimbursement purposes.

IN USE verify the billing category

identity match

Confirmation that a person corresponds to the correct record.

IN USE establish the identity match

name pronunciation

The way a person's name is spoken.

IN USE check name pronunciation

Precision check

Using Alex does not mean silently replacing verified identifiers or creating another record. Follow the clinic process for recorded preferences and corrections. Respectful address supports accurate identification; it does not remove the need for it.

Phrases you can actually use

C Notice the language / 1 of 2

USE THE PREFERRED NAME

"Thank you, Alex; I will use that name."

ACKNOWLEDGE THE EXPECTATION

"You expected to discuss a recent concern."

AVOID A CLINICAL DECISION

"I cannot decide whether this appointment type meets that need."

PRESERVE IDENTIFICATION

"We will keep the verified record linked correctly."

STATE THE SCHEDULE

"The schedule lists an annual review."

NAME THE DIFFERENCE

"The recorded visit type and your expectation are not the same."

OFFER A NAMED ROUTE

"Nurse Dana can clarify the visit plan."

AVOID BLAME

"Let us clarify the plan before assuming how the mismatch happened."

Lists

Attributes the category to the schedule rather than presenting it as a clinical decision.

You expected

Acknowledges the patient's understanding without assigning fault.

Can clarify

Offers a next step without promising a clinical outcome.

Phrases you can actually use

C Notice the language / 2 of 2

SEPARATE BILLING

"I cannot determine charges from this appointment label alone."

PROTECT PRIVACY

"We can use the appropriate setting for personal details."

ASK FOR CLARIFICATION

"Dana, can you clarify the visit plan with Alex?"

AVOID DUPLICATE RECORDS

"We should use the correctly matched existing record."

KEEP THE CONCERN VISIBLE

"I will include your stated concern in the handoff."

DO NOT PROMISE TREATMENT

"I cannot promise a particular clinical outcome."

CONFIRM THE PREFERENCE

"The verified record says Alexandra; the preferred spoken name is Alex."

CLOSE THE LOOP

"I will explain the mismatch to Dana so you do not have to start from nothing."

Before assuming

Stops blame from replacing a factual check.

Linked correctly

Keeps respectful address and accurate identification together.

From the label alone

Limits conclusions about billing or suitability.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which handoff preserves both facts?

- A** The schedule says annual review; Alex expected to discuss a recent concern.
- B** Alex booked incorrectly and must leave.
- C** Annual review guarantees treatment of every concern.
- D** The record is definitely for another person.

2. What does using Alex change here?

- A** The billing category automatically
- B** The respectful form of spoken address, not the verified identity match
- C** The need for all patient identifiers
- D** The clinical urgency of the concern

3. Which reassurance is unsupported?

- A** The schedule says annual review.
- B** Sam cannot decide clinical suitability.
- C** This visit will definitely resolve the concern at no additional cost.
- D** Dana can clarify the plan.

4. Which action is appropriate?

- A** Pass the mismatch to Dana while keeping the correctly matched record.
- B** Create another record because the patient prefers Alex.
- C** Dismiss the concern because it is not in the schedule label.
- D** Change the clinical plan independently.

Answers and explanations: [page 85](#).

The schedule and the expectation

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

preferred name / clinical handoff / presenting concern / visit plan / appointment type / identity match / billing category / patient expectation / clinical suitability / scheduled purpose

- 01 **Alex:** I am here for my appointment, but I noticed the reminder says annual review. I thought this was the visit for the concern I called about.
- 02 **Sam:** The ¹ _____ is an annual review. Thank you for explaining what you expected; I will ask nurse Dana to clarify the plan.
- 03 **Alex:** Before we go further, please call me Alex. Alexandra is the name in my records, but I use Alex in conversation and at work.
- 04 **Sam:** Of course, Alex. Your ² _____ is clear, and we will keep the verified record correctly linked. I am not creating another record because you use a shorter name.
- 05 **Alex:** Good. I worry that the appointment category means nobody will hear the reason I actually came. Do I need to book something else?
- 06 **Sam:** I cannot decide ³ _____ from the schedule label. Dana can clarify how the visit should address your concern before we assume that another booking is required.
- 07 **Alex:** I do not want to explain personal details here in front of everyone. Can the question reach Dana without a discussion across reception?
- 08 **Sam:** Yes. I can identify the mismatch and use the appropriate private route for the ⁴ _____. We do not need to announce personal details to the waiting room.
- 09 **Alex:** Will you tell Dana that I expected this to be the concern visit, rather than just saying that I am unhappy about the reminder?
- 10 **Sam:** I will preserve that ⁵ _____. The handoff will say the schedule lists an annual review and you expected a discussion of a recent concern.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Alex:** Is this a billing problem too? I have heard different visit categories can mean different charges, and I do not want a surprise.
- 12 **Sam:** A ⁶ _____ is another question to clarify through the proper process. I cannot promise a charge or coverage outcome from the label alone.
- 13 **Alex:** Please check before anyone promises it is covered. I took time off for this visit, and I need to understand what will actually happen today.
- 14 **Sam:** I can confirm the schedule and your expectation, then request a ⁷ _____ to Dana. I cannot promise how the clinician will resolve the concern.
- 15 **Alex:** Please also make sure the team uses Alex. Sometimes one person agrees, but the preference disappears as soon as I move to another room.
- 16 **Sam:** I will handle that preference through our record process while preserving the ⁸ _____. The name used in conversation and the correctly matched record need not conflict.
- 17 **Alex:** Thank you. I do not need you to change the appointment yourself; I need someone qualified to clarify what today's visit can include.
- 18 **Sam:** That is the ⁹ _____ question for Dana. I will not turn the appointment label into a refusal or a guarantee before that discussion.
- 19 **Alex:** All right. Can you let Dana know I am here? I would like to settle the visit plan before I explain everything again.
- 20 **Sam:** Correct. I will record the ¹⁰ _____ accurately and pass on your expectation. We will keep the two facts clear while Dana clarifies the next step.

ANSWERS / EXPLANATIONS: PAGE 86

1. scheduled purpose | 2. preferred name | 3. clinical suitability | 4. presenting concern | 5. patient expectation | 6. billing category | 7. clinical handoff | 8. identity match | 9. visit plan | 10. appointment type

Clarify a different appointment expectation

F Say it

The schedule says follow-up. Patient Morgan expected an initial consultation. Assistant Lee cannot decide suitability. Nurse Inez is available to clarify the plan.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

Inez / initial consultation / follow-up / established

Lee: "The recorded visit is a ¹ _____."

Morgan: "I expected an ² _____."

Lee: "Nurse ³ _____ can clarify the plan."

Lee: "Clinical suitability is not yet ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. follow-up | 2. initial consultation | 3. Inez | 4. established

Rehearse with a fixed script

1. Read Alex and Sam's corrected exchange. Stress annual review, recent concern, and Dana; keep the patient concern separate from the booking label.
2. Switch roles. Repeat the privacy request and preferred-name exchange without changing the verified identity facts.
3. Complete Morgan's four-line exchange, check the key, then read it aloud with follow-up and initial consultation clearly contrasted.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Report, observation, assessment

A Read the situation

Give a concise patient report that separates the patient words, observed behavior, and unassessed clinical questions.

During rooming, patient Pat says, "I have felt tired since Monday." Assistant Rosa observed Pat answering questions while seated. Rosa has not assessed the cause or severity, and no measurement values are supplied. Nurse Dana is receiving the report. The conversation must preserve the onset as reported by Pat, state the limited observation, and avoid filling missing fields with normal values. Sitting and answering questions do not establish that the concern is mild or safe to defer. Actual urgency must follow the clinic clinical process.

PEOPLE / Rosa: Medical assistant | Dana: Receiving nurse

NEUTRAL IS NOT DISMISSIVE

A factual report can be both concise and attentive. Use the patient words and name the limits of the observation. Avoid vague labels such as fine or difficult, which replace useful information with an impression and can distort the clinical handoff.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What onset did Pat report?

- A This morning confirmed by testing
- C Monday

- B No symptoms ever
- D A date measured by Rosa

2. What did Rosa observe?

- A A completed severity assessment
- C Normal blood pressure

- B Pat answering questions while seated
- D A diagnosed cause of tiredness

3. What should remain unassessed in the report?

- A Cause and severity
- C Whether Dana is receiving the report

- B Whether Pat spoke
- D Whether Monday was mentioned

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

rooming

Preparing a patient for the clinical encounter under the clinic process.

IN USE complete the rooming process

patient-reported symptom

Experience described by the patient rather than independently observed.

IN USE attribute a patient-reported symptom

onset

When a symptom or condition began.

IN USE clarify the reported onset

duration

Length of time an experience or condition has continued.

IN USE document symptom duration

severity

Degree or intensity of a condition or symptom.

IN USE assess severity

observation

Something directly noticed and described without inventing its cause.

IN USE record an observation

subjective information

Information based on a person's reported experience.

IN USE attribute subjective information

objective finding

Observed or measured finding, with its source and context.

IN USE document an objective finding

baseline

Usual or earlier state used for comparison.

IN USE establish the baseline

change from baseline

Difference from a known usual or previous state.

IN USE report a change from baseline

associated symptom

Additional symptom occurring with the main concern.

IN USE ask about associated symptoms

functional impact

Effect on a person's ability to perform ordinary activities.

IN USE describe functional impact

Precision check

Not reported does not mean absent, and not measured does not mean normal. A pertinent negative requires a relevant question or assessment. Do not populate a record from expectation or convert an empty field into reassurance.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

vital signs

Physiological measurements such as pulse and temperature.

IN USE record measured vital signs

blood pressure

Pressure exerted by blood within the circulation, measured with relevant units.

IN USE document blood pressure

pulse rate

Number of palpable arterial pulses per unit of time.

IN USE measure pulse rate

respiratory rate

Number of breaths per unit of time.

IN USE record respiratory rate

oxygen saturation

Percentage of hemoglobin oxygen-binding sites occupied by oxygen, estimated or measured by an appropriate method.

IN USE report oxygen saturation

temperature

Measure of how hot or cold the body or another object is.

IN USE record the measured temperature

unit of measurement

Standard used to express a measured quantity.

IN USE include the unit of measurement

measurement time

Time at which a measurement was actually taken.

IN USE preserve the measurement time

clinical assessment

Qualified evaluation of health information and the patient's condition.

IN USE request clinical assessment

unassessed

Not evaluated through the relevant assessment.

IN USE mark severity as unassessed

pertinent negative

Relevant absence established by asking or assessing, not by silence.

IN USE document a pertinent negative

source attribution

Identification of where a statement or finding came from.

IN USE preserve source attribution

Precision check

Pat answering questions while seated is a limited observation. It does not establish alertness on a formal scale, normal vital signs, a cause of tiredness, or a severity rating. Preserve what was actually observed and leave interpretation with the appropriate clinician.

Phrases you can actually use

C Notice the language / 1 of 2

ATTRIBUTE THE WORDS

"Pat reports feeling tired since Monday."

DESCRIBE THE OBSERVATION

"I observed Pat answering questions while seated."

DO NOT INVENT VALUES

"No measurement values are available in this report."

KEEP ONSET ATTRIBUTED

"Monday is the onset Pat reported."

QUOTE ACCURATELY

"Pat said, "I have felt tired since Monday.""

NAME THE LIMIT

"I have not assessed the cause or severity."

SEPARATE SOURCES

"That is the patient report, not a measured finding."

AVOID A VAGUE REASSURANCE

"I cannot describe the concern as mild from that observation."

Pat reports

Keeps the patient's experience distinct from an assistant's clinical conclusion.

I observed

Identifies a firsthand observation and its source.

Since Monday

Gives the reported starting point without inventing a date or duration.

Phrases you can actually use

C Notice the language / 2 of 2

STATE WHAT IS MISSING

"The relevant clinical assessment is still needed."

KEEP THE HANDOFF CONCISE

"I will give the report, observation, and outstanding assessment separately."

REFER THE JUDGMENT

"Dana, please assess the clinical concern through our process."

CONFIRM RECEIPT

"Have you received Pat's report and the limits of my observation?"

DO NOT IMPLY ABSENCE

"An unreported symptom is not automatically absent."

USE UNITS ACCURATELY

"Any actual measurement needs its correct value, unit, and context."

PRESERVE UNCERTAINTY

"I do not have a verified explanation for the tiredness."

CLOSE FACTUALLY

"The cause and severity remain unassessed in my report."

Have not assessed

Describes the actual evidence limit, not a negative result.

Not automatically absent

Prevents a blank field from becoming a clinical finding.

Separately

Helps a receiving professional distinguish evidence types quickly.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which report is supported?

- A** Pat is safe to wait because Pat can speak.
- B** Pat reports tiredness since Monday; I observed Pat answering questions while seated.
- C** Pat has a mild condition caused by poor sleep.
- D** All vital signs are normal.

2. What belongs in the temperature field when no measurement is available?

- A** A normal category inferred from Pat answering questions
- B** A numeric value inferred from the reported tiredness
- C** No invented numeric value
- D** A previous visit's value entered as today's reading

3. Why is "Pat is fine" inadequate?

- A** It preserves the patient quotation.
- B** It identifies a measured oxygen level.
- C** It substitutes a broad conclusion for the limited observation.
- D** It gives the exact onset.

4. Which statement about other symptoms is accurate?

- A** Every blank field can be completed as normal.
- B** No other symptoms are supplied; that does not prove their absence.
- C** Every unmentioned symptom was denied.
- D** Pat has no associated symptoms.

Answers and explanations: [page 87](#).

Report, observation, assessment

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

pertinent negative / severity / unassessed / Vital signs / onset / observation / source attribution / Clinical assessment / baseline / patient-reported symptom

- 01 **Rosa:** Dana, I have Pat's rooming report. Pat said, "I have felt tired since Monday." I want to keep that separate from what I observed.
- 02 **Dana:** Please give the ¹ _____ clearly. What came from Pat, what did you directly observe, and what has not been assessed?
- 03 **Rosa:** The tiredness and Monday starting point came from Pat. I observed Pat answering questions while seated. I have not assessed the cause or severity.
- 04 **Dana:** Good. The reported ² _____ is Monday. Do not replace it with a guessed date or calculate a duration from an assumed calendar.
- 05 **Rosa:** The handoff draft says looks fine. I will replace that with what I saw: Pat was seated and answering my questions.
- 06 **Dana:** Exactly. Keep the ³ _____ specific. Answering questions while seated does not establish the severity of tiredness or whether a concern can safely wait.
- 07 **Rosa:** I also need to flag the blank measurement fields. I have no values to report; those fields should not be read as normal findings.
- 08 **Dana:** Correct. ⁴ _____ need actual values and appropriate context. A missing measurement cannot be converted into normal by choosing a familiar number.
- 09 **Rosa:** Should I write that Pat denied other symptoms? None are included in the information I am handing over, but I have not established their absence.
- 10 **Dana:** No. A ⁵ _____ must come from the relevant question or assessment. Lack of a statement in this report is not a denial.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Rosa:** I will keep the original wording in quotation marks when I use it exactly. If I summarize, I will still identify Pat as the source.
- 12 **Dana:** That preserves the ⁶ _____. Do not translate tired into a specific diagnosis or imply that Pat used a technical term they did not actually say.
- 13 **Rosa:** I also cannot say this is a change from Pat's normal condition unless I have an established usual state to compare it with.
- 14 **Dana:** Right. A ⁷ _____ is not something we should invent. Your report can remain useful without adding a comparison that has not been obtained.
- 15 **Rosa:** The handoff can be short: Pat reports tiredness since Monday; observed answering questions while seated; cause and severity not assessed by me.
- 16 **Dana:** That clearly identifies what is ⁸ _____. It does not minimize the concern, and it leaves the clinical questions visible for the proper assessment.
- 17 **Rosa:** Have you received those points? I do not want the phrase answering questions to be passed on as a formal severity rating later.
- 18 **Dana:** I have received the report and its limits. ⁹ _____ is separate, and the concern will follow our clinical process rather than an administrative assumption.
- 19 **Rosa:** I will record the report accordingly and preserve what I actually observed. I will not add a measurement, diagnosis, or assurance about waiting.
- 20 **Dana:** Thank you. Keep the ¹⁰ _____ question open in that record. Clear limits help the receiving team know what still requires attention.

ANSWERS / EXPLANATIONS: PAGE 88

1. source attribution | 2. onset | 3. observation | 4. Vital signs | 5. pertinent negative | 6. patient-reported symptom | 7. baseline | 8. unassessed | 9. Clinical assessment | 10. severity

Attribute a different symptom report

F Say it

Patient Lee reports dizziness since breakfast. Assistant Noor observed Lee speaking while seated. No measurements or cause assessment are supplied. Nurse Inez receives the report.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

dizziness / unassessed / seated / breakfast

Noor: "Lee reports ¹ _____ since breakfast."

Inez: "The reported onset is ² _____."

Noor: "I observed Lee speaking while ³ _____."

Inez: "The cause remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. dizziness | 2. breakfast | 3. seated | 4. unassessed

Rehearse with a fixed script

1. Read Rosa and Dana's corrected report. Contrast Pat reports with I observed; keep the reported onset attached to Pat.
2. Switch roles. Repeat the unmeasured-vitals and pertinent-negative exchanges without adding a normal finding.
3. Complete the second patient report, check the key, and read the patient's words separately from the observation.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Two sources, one unresolved entry

A Read the situation

03

Document a reported medication change without treating an unverified history as a new clinical instruction.

Patient Ren reports that another clinic stopped one medicine. The local medication list still includes it. Ren does not have the outside instruction available. Assistant Talia cannot verify discontinuation, change the list as a clinical decision, or tell Ren to start, stop, or restart treatment. Nurse Dana can review the discrepancy with the clinician. The immediate communication task is to record the patient report and the conflicting local entry with clear sources, then connect the question with Dana. No medicine name, dose, or timing decision is supplied.

PEOPLE / Ren: Patient | Talia: Medical assistant

A DISCREPANCY IS NOT A CREDIBILITY CONTEST

Do not make the patient defend their memory against the computer. Acknowledge the report and preserve both sources for review. Respect does not require treating an unverified instruction as confirmed, and careful verification does not require dismissing the patient.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Ren report?

- A** A new verified prescription has arrived here.
- B** The local clinician has confirmed a restart.
- C** The medication list is already reconciled.
- D** Another clinic stopped one medicine.

2. What conflicts with that report?

- A** The medicine remains on the local list.
- B** A supplied laboratory result
- C** An established allergy reaction
- D** A confirmed replacement dose

3. What can Dana do?

- A** Guarantee that Ren should restart treatment
- B** Prove the outside clinic was wrong without evidence
- C** Treat the local list as a new prescription automatically
- D** Review the discrepancy with the clinician

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

medication history

Account of medicines used or reported, with relevant sources and timing.

IN USE obtain a medication history

medication reconciliation

Process of comparing medication information and resolving discrepancies appropriately.

IN USE support medication reconciliation

local medication list

Medicine record held by the current organization.

IN USE review the local medication list

outside instruction

Direction originating from another healthcare source.

IN USE verify the outside instruction

discontinuation

Ending a medicine or order through the relevant clinical process.

IN USE confirm discontinuation

patient report

Information supplied by the patient about their experience or history.

IN USE document the patient report

active entry

Record item currently marked active in a system.

IN USE verify an active entry

historical entry

Record of earlier information rather than necessarily current treatment.

IN USE distinguish a historical entry

duplicate entry

Repeated record item that may need clarification.

IN USE investigate a duplicate entry

omission

Information missing from a list or account.

IN USE identify a possible omission

discrepancy

Difference between sources requiring clarification.

IN USE flag a medication discrepancy

prescribing clinician

Professional authorized to prescribe the relevant medicine.

IN USE contact the prescribing clinician

Precision check

An active entry can reflect the state of a record rather than a verified current instruction. Conversely, a patient report deserves documentation but does not automatically complete reconciliation. Preserve both until the authorized review resolves the discrepancy.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

dose

Amount intended to be taken or administered at one time.

IN USE verify the prescribed dose

strength

Amount of active ingredient in a specified unit of product.

IN USE distinguish strength from dose

dosage form

Physical form of a medicine, such as tablet or solution.

IN USE confirm the dosage form

route

Way a medicine is taken or administered.

IN USE verify the route

frequency

How often a medicine is taken or administered.

IN USE clarify the frequency

as-needed instruction

Direction allowing use under specified conditions rather than a fixed schedule alone.

IN USE verify the as-needed instruction

over-the-counter medicine

Medicine available without a prescription under applicable rules.

IN USE record over-the-counter medicines

dietary supplement

Product used to supplement dietary intake, subject to relevant regulation.

IN USE ask about dietary supplements

allergy history

Account of suspected or confirmed allergic reactions with relevant details.

IN USE document allergy history

adverse reaction

Harmful or unwanted response associated with a medicine or other exposure.

IN USE report an adverse reaction

adherence

Extent to which medicine use corresponds with the agreed plan.

IN USE discuss adherence without blame

verification source

Person, document, or system used to confirm information.

IN USE identify the verification source

Precision check

A history question is not permission to advise starting, stopping, or restarting a medicine. Dose, strength, route, and frequency describe different aspects of an instruction. Do not fill any of them from memory or from a similar product.

Phrases you can actually use

C Notice the language / 1 of 2

03

ACKNOWLEDGE THE REPORT

"You report that another clinic stopped this medicine."

NAME THE DISCREPANCY

"Those two sources need to be reconciled."

KEEP ATTRIBUTION

"I will record that this information came from you."

GIVE THE REVIEW ROUTE

"Dana can review the discrepancy with the clinician."

STATE THE LOCAL SOURCE

"The local medication list still includes it."

AVOID JUDGING MEMORY

"I will record what you remember and ask the clinical team to check the difference."

IDENTIFY THE MISSING EVIDENCE

"The outside instruction is not available here."

AVOID TREATMENT ADVICE

"I cannot tell you to start, stop, or restart it."

You report

Attributes the change without dismissing it or confirming it prematurely.

Still includes

Describes the local record rather than asserting that treatment must continue.

Those two sources

Treats the problem as reconciliation instead of a contest over credibility.

Phrases you can actually use

C Notice the language / 2 of 2

03

DO NOT SILENTLY DELETE

"We need the appropriate review before treating the entry as resolved."

SEPARATE STRENGTH AND DOSE

"The product strength and prescribed amount are different details."

DO NOT INVENT A REACTION

"No allergy or adverse reaction is established by this discrepancy."

CONFIRM THE HANDOFF

"Dana will receive both the patient report and the local-list difference."

PRESERVE THE DETAILS

"I will keep the original source and the reported change clear."

INCLUDE NONPRESCRIPTION USE

"Relevant nonprescription medicines and supplements also belong in an accurate history."

KEEP URGENCY SEPARATE

"A current medicine concern needs the appropriate clinical route."

CLOSE WITHOUT A NEW INSTRUCTION

"This conversation records the question; it does not change your treatment."

Not available here

Names the evidence gap without claiming the outside instruction never existed.

Start, stop, or restart

Makes the clinical boundary explicit in ordinary language.

Does not change

Prevents a history discussion from being mistaken for prescribing advice.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which note is accurate?

- A** Discontinuation verified from the patient's memory alone.
- B** Patient is nonadherent and the outside clinic is wrong.
- C** Restart approved because the computer says active.
- D** Patient reports outside discontinuation; local list still includes medicine; instruction unavailable; referred to Dana.

2. What does the active local entry establish by itself?

- A** The patient must take it now.
- B** The outside report is false.
- C** Reconciliation is complete.
- D** The medicine remains listed locally, not that a new instruction has been verified.

3. Which reply crosses Talia's stated role?

- A** I cannot verify the outside instruction here.
- B** Restart it until we hear otherwise.
- C** Dana can review the discrepancy.
- D** I will record the source of your report.

4. Which additional conclusion is unsupported?

- A** The sources conflict.
- B** The outside instruction is unavailable.
- C** The clinician review is still needed.
- D** The discrepancy proves an allergy.

Answers and explanations: [page 89](#).

Two sources, one unresolved entry

E Conversations / turns 1-10

03

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

verification source / discontinuation / patient report / outside instruction / discrepancy /
Medication reconciliation / local medication list / active entry / adherence / Strength

- 01 **Ren:** Your list still shows a medicine that another clinic told me to stop. I do not have their instruction with me, but I remember the conversation.
- 02 **Talia:** Thank you for telling me. I will record your ¹ _____ and the difference in our list. I am not going to dismiss it because the computer still shows an entry.
- 03 **Ren:** Does the computer mean I should be taking it after all? I do not want two clinics giving me opposite directions without explaining why.
- 04 **Talia:** The ² _____ is a record we need to review. It does not let me give you a new instruction to take, stop, or restart a medicine.
- 05 **Ren:** Then please do not remove it and say everything is sorted either. I want the right person to check what the other clinic actually intended.
- 06 **Talia:** Exactly. We need the ³ _____ and appropriate review. Nurse Dana can take the discrepancy to the clinician; I cannot verify the change myself from this conversation.
- 07 **Ren:** I appreciate that. Sometimes questions about medicines sound as though I am being accused of not following instructions, even when I am trying to explain a change.
- 08 **Talia:** I will not label this an ⁴ _____ problem without evidence. The immediate issue is that your report and our entry differ, and we need to clarify the plan.
- 09 **Ren:** I do not have the other clinic's instructions with me. Can you help get them checked? I am worried about being given conflicting advice again.
- 10 **Talia:** Yes. The ⁵ _____ is still to be established through the review. I will distinguish your report from a verified clinical decision.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Ren:** The medicine being marked active is what confused me. Is that label always current, or can the record need updating after another clinic makes a change?
- 12 **Talia:** An ⁶ _____ tells us how the item is marked. It does not settle this conflicting information, so we must not treat that label as the answer.
- 13 **Ren:** I also do not have the exact strength or dose in front of me. I would rather say that than give you a number from memory.
- 14 **Talia:** That is helpful. ⁷ _____ and dose are different details, and neither should be guessed. We will preserve what is known and identify what requires verification.
- 15 **Ren:** Can Dana speak with me about it? The list worries me because I still need to know which instruction I should follow.
- 16 **Talia:** It is a ⁸ _____ needing the appropriate review. I will pass on both sources and the missing instruction rather than silently choose one over the other.
- 17 **Ren:** Please make sure any current question about what I should do reaches the clinical team. I do not want the record discussion mistaken for actual advice.
- 18 **Talia:** I will route that clinical question appropriately. ⁹ _____ and clinical guidance need the proper owner, and recording a history does not authorize me to change treatment.
- 19 **Ren:** Then I understand the position: I reported an outside change, your list still includes the medicine, and Dana will review the difference with the clinician.
- 20 **Talia:** Correct. The reported ¹⁰ _____ is not yet verified here. I will keep the source clear, preserve the question, and connect the review with Dana.

ANSWERS / EXPLANATIONS: PAGE 90

1. patient report | 2. local medication list | 3. outside instruction | 4. adherence | 5. verification source | 6. active entry | 7. Strength | 8. discrepancy | 9. Medication reconciliation | 10. discontinuation

Report a different medication-list change

F Say it

Patient Ellis reports an outside dose change. The local record is unchanged, and the outside instruction is unavailable. Nurse Priya accepts the query. No new dose is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

dose / Priya / unchanged / unavailable

Assistant: "Ellis reports an outside ¹ _____ change."

Nurse: "The local record remains ² _____."

Assistant: "The outside instruction is ³ _____."

Nurse: "The receiving owner is ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. dose | 2. unchanged | 3. unavailable | 4. Priya

Rehearse with a fixed script

1. Read Ren's question and Talia's responses using the checked answers. Stress reports, active, unavailable, and review.
2. Switch roles. Repeat the exchange about the outside instruction without adding a dose or restart recommendation.
3. Complete the Ellis transfer, check the key, then read it aloud with Priya clearly named as the receiving owner.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Two times, two events

A Read the situation

04

Preserve collection and arrival times accurately when a colleague proposes an unnecessary record change.

A requisition records specimen collection at 09:20. The courier sheet records arrival at 10:05. Colleague Ben proposes replacing 09:20 with 10:05 so the paperwork looks consistent. Assistant Imani notices that the fields describe different events. No patient-identity mismatch, specimen-quality problem, or test result is stated. The pair must retain the event labels and source records, clarify any genuinely ambiguous field through the laboratory process, and avoid creating a false collection time. The 45-minute interval alone does not establish specimen acceptability.

PEOPLE / Ben: Clinic colleague | Imani: Medical assistant

QUESTION THE FIELD, NOT THE COLLEAGUE

A correction request can be challenged without making the exchange personal. Name the field and event that would change, then propose a read-back. Two different values can both be correct when they refer to different events.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which event occurred at 09:20 according to the requisition?

A Courier arrival

B Result authorization

C Patient discharge

D Collection

2. What does 10:05 describe?

A A revised collection time

B A confirmed rejection

C A reported test result

D Arrival on the courier sheet

3. What does the interval establish by itself?

A A 45-minute difference between the recorded events

B Specimen acceptability for every test

C A proven transport failure

D A diagnostic conclusion

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

requisition

Request document specifying laboratory testing and relevant information.

IN USE check the requisition

specimen

Material collected for examination or testing.

IN USE identify the specimen

collection time

Time the specimen was obtained from its source.

IN USE record the collection time

arrival time

Time an item reached the stated destination.

IN USE verify the arrival time

receipt time

Time the receiving service recorded receiving the specimen.

IN USE document the receipt time

courier sheet

Transport record containing delivery or movement information.

IN USE reconcile the courier sheet

accession number

Identifier assigned by a laboratory to track a case or specimen.

IN USE confirm the accession number

specimen label

Information attached to the specimen container for identification.

IN USE verify the specimen label

specimen source

Body site or material origin relevant to the test.

IN USE record the specimen source

collection method

Technique by which a specimen was obtained.

IN USE document the collection method

transport interval

Time between specified transport-related events.

IN USE calculate the transport interval

preanalytical phase

Steps before analysis that can affect laboratory testing.

IN USE review the preanalytical phase

Precision check

Collection, courier arrival, laboratory receipt, analysis, and result release are distinct events. Do not assume that arrival on a transport sheet means formal laboratory receipt unless the record defines it that way. Always preserve the stated event label.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

analytical phase

Stage in which the laboratory examination or measurement is performed.

IN USE distinguish the analytical phase

postanalytical phase

Steps after analysis, including review and reporting.

IN USE describe the postanalytical phase

acceptance criteria

Requirements for accepting a specimen or process.

IN USE check acceptance criteria

rejection criteria

Conditions under which a specimen cannot be accepted for the intended test.

IN USE apply rejection criteria

specimen integrity

Condition of the specimen relevant to reliable testing.

IN USE assess specimen integrity

hemolysis

Breakdown of red blood cells that can affect some tests.

IN USE report hemolysis

insufficient quantity

Amount below what is required for the intended test.

IN USE report insufficient quantity

transport medium

Material used to preserve or support a specimen during movement.

IN USE verify the transport medium

temperature condition

Required or recorded temperature circumstances.

IN USE document temperature conditions

event label

Description of the event to which a time or value belongs.

IN USE preserve the event label

record amendment

Traceable correction or addition through the approved record process.

IN USE make a record amendment

turnaround time

Interval between defined start and completion events for a service.

IN USE define the turnaround time

Precision check

A 45-minute interval is not itself a specimen-quality verdict. Requirements depend on the specimen, test, container, handling, and actual laboratory procedure. This lesson practices document language, not collection, transport, acceptance, or rejection procedures.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE FIRST FIELD

"The requisition says collection at 09:20."

NAME THE DISTINCTION

"Those times describe different events."

KEEP BOTH FACTS

"We should preserve each time with its event label."

READ BACK THE SEQUENCE

"Collected at 09:20; arrival recorded at 10:05."

IDENTIFY THE SECOND FIELD

"The courier sheet says arrival at 10:05."

QUESTION THE PROPOSED CHANGE

"Why would an arrival time replace a collection time?"

AVOID COSMETIC CORRECTION

"Matching numbers would make the record less accurate here."

CLARIFY AN AMBIGUOUS FIELD

"Let us ask the laboratory which event this field requires."

Says collection

Anchors the value to the source and event.

Different events

Explains why numerical disagreement is not necessarily an error.

Would replace

Makes the consequence of the proposed edit explicit.

Phrases you can actually use

C Notice the language / 2 of 2

DO NOT INVENT A RECEIPT EVENT

"The sheet says arrival; it does not establish every later laboratory step."

NAME MISSING CRITERIA

"We do not have the test-specific acceptance criteria here."

PRESERVE THE SOURCE

"Retain the original requisition and courier references."

DO NOT INFER A RESULT

"The timing records do not tell us the test result."

SEPARATE CALCULATION AND JUDGMENT

"The interval is 45 minutes, not an acceptability decision."

KEEP IDENTIFIERS SEPARATE

"No identity discrepancy is established by these different times."

USE A TRACEABLE PROCESS

"Any genuine correction needs the approved amendment process."

CLOSE ACCURATELY

"Both entries can remain accurate when their event labels are preserved."

Here

Limits the conclusion to the supplied records, not every future discrepancy.

Not an acceptability decision

Separates a calculation from a laboratory judgment.

Any genuine correction

Leaves room for real errors without manufacturing one.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which combined entry preserves the facts?

- A** Collection 10:05; arrival 10:05
- B** Collection and arrival both unknown
- C** Test authorized at 09:20
- D** Collection 09:20; arrival 10:05

2. Why should 09:20 not be replaced merely for consistency?

- A** All timestamps must always be identical.
- B** Arrival is a more important word than collection.
- C** The test is known to have failed.
- D** The replacement would change a collection event into an arrival time.

3. Which conclusion is unsupported?

- A** The records describe two events.
- B** The interval is 45 minutes.
- C** No result is supplied.
- D** The specimen is acceptable because only 45 minutes elapsed.

4. If a destination field is unclear, what is the next step?

- A** Clarify the required event with the laboratory through the approved process.
- B** Copy whichever time is later.
- C** Replace all original values.
- D** Invent an accession number.

Answers and explanations: [page 91](#).

Two times, two events

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

collection time / Receipt time / arrival time / record amendment / requisition / Turnaround time / event label / transport interval / Acceptance criteria / specimen label

- 01 **Ben:** I have two different times on this paperwork. The requisition says 09:20, but the courier sheet says 10:05. Should I make them match?
- 02 **Imani:** First check the ¹ _____. The requisition records collection, while the courier sheet records arrival. Those are not two competing values for the same event.
- 03 **Ben:** So replacing the earlier time would not simply clean up the paperwork. It would say the specimen was collected later than the requisition records.
- 04 **Imani:** Exactly. The ² _____ is 09:20 in that source. Keeping the record accurate matters more than making two boxes look identical.
- 05 **Ben:** Let me read it back: collected at nine twenty; arrival at ten oh five. The difference is forty-five minutes, but each time has its own meaning.
- 06 **Imani:** That preserves the ³ _____ correctly. We also should not rename arrival as a later laboratory event unless the record actually defines it that way.
- 07 **Ben:** I was about to enter ten oh five in a field labeled received. I should check what that field means before assuming the courier entry is enough.
- 08 **Imani:** Yes. ⁴ _____ depends on the receiving record and process. Ask the laboratory which event the destination field requires rather than copying a nearby value.
- 09 **Ben:** The patient details match in these records. I was only looking at the two times; I do not have evidence of a labeling problem.
- 10 **Imani:** That would be a separate ⁵ _____ or identification question. No identity discrepancy is established here, so we should not turn an event-time difference into a patient mismatch.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Ben:** A colleague called that a quick delivery and marked the specimen acceptable. We have the journey time, but have we checked the requirements for this test?
- 12 **Imani:** It gives the ⁶ _____ between these entries. It does not establish acceptability without the relevant specimen, test, handling conditions, and actual laboratory requirements.
- 13 **Ben:** Then I will avoid writing acceptable or rejected. Neither result follows just from the times we have read on these two documents.
- 14 **Imani:** Correct. ⁷ _____ belong to the real laboratory process. A language clarification about timestamps is not a quality decision.
- 15 **Ben:** If the laboratory later confirms that a field genuinely was entered incorrectly, we can correct it through the proper process while keeping the history clear.
- 16 **Imani:** That would be a ⁸ _____. It should preserve what was changed and why under our requirements, rather than silently overwrite the original source.
- 17 **Ben:** I also should not tell the patient that these times predict when a result will be ready. Collection-to-arrival is not the entire testing process.
- 18 **Imani:** Exactly. ⁹ _____ must have defined start and end points. These entries do not supply an analysis completion or result-release time.
- 19 **Ben:** I will keep both source references, preserve 09:20 as collection and 10:05 as arrival, and clarify the receiving field before entering anything there.
- 20 **Imani:** Good. The ¹⁰ _____ and courier sheet can both be accurate. Our task is to carry their meanings forward without creating a false match.

ANSWERS / EXPLANATIONS: PAGE 92

1. event label | 2. collection time | 3. arrival time | 4. Receipt time | 5. specimen label | 6. transport interval | 7. Acceptance criteria | 8. record amendment | 9. Turnaround time | 10. requisition

Keep a second pair of events distinct

F Say it

The collection record says 08:40. The courier sheet says arrival 09:15. The destination field meaning is unclear. No acceptability decision is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

09:15 / 08:40 / distinct / clarification

Assistant: "Collection was recorded at ¹ _____."

Colleague: "Arrival was recorded at ² _____."

Assistant: "The event labels must remain ³ _____."

Colleague: "The destination field needs ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. 08:40 | 2. 09:15 | 3. distinct | 4. clarification

Rehearse with a fixed script

1. Read Ben and Imani's corrected dialogue. Say nine twenty and ten oh five clearly, with collection and arrival attached.
2. Switch roles. Repeat the received-field clarification and the distinction between elapsed time and acceptability.
3. Complete the 08:40 and 09:15 transfer, check the key, and read it aloud without merging the two events.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Friday is a status check

A Read the situation

Explain referral stages and a named follow-up date without accidentally confirming an appointment.

05

Referral F8 was sent Monday and received by the specialist office Tuesday. The office reports that it is under review; no appointment is booked. Coordinator Lia has agreed to check its status Friday. Patient Morgan believes receipt means an appointment on Friday. Assistant Omar must clarify the actual sequence, distinguish a status check from a booking, and identify Lia as the follow-up owner. No review deadline, appointment date, coverage decision, or reason for the review duration is supplied. Clinical concerns require the clinical route, not a wait for this administrative check.

PEOPLE / Morgan: Patient | Omar: Medical assistant

EXPLAIN THE STAGE IN ORDINARY LANGUAGE

A word such as received may sound final to someone outside the clinic. Give the plain-language meaning and the next known action. Repeating under review without explanation can sound evasive; inventing a likely appointment can be misleading.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What happened Tuesday?

- A** The patient attended a specialist appointment.
- B** Lia completed the Friday check.
- C** A coverage decision was issued.
- D** The specialist office received referral F8.

2. What is Friday for?

- A** Lia's status check
- B** A confirmed patient appointment
- C** A guaranteed review deadline
- D** A promised test result

3. What remains unconfirmed?

- A** An appointment date
- B** That the referral was sent Monday
- C** That receipt occurred Tuesday
- D** Lia's ownership of the status check

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

referral

Request for another service or professional to assess or provide care.

IN USE send a referral

referring clinician

Professional initiating a referral.

IN USE identify the referring clinician

receiving service

Organization or team to which a referral is sent.

IN USE contact the receiving service

specialist office

Practice or department providing care in a particular specialty.

IN USE check with the specialist office

referral identifier

Reference used to track a particular referral.

IN USE quote the referral identifier

sent status

Record that a referral or message was transmitted.

IN USE verify the sent status

receipt acknowledgment

Confirmation that the receiving organization obtained the referral.

IN USE obtain a receipt acknowledgment

under review

Being assessed through a process before a further decision or action.

IN USE explain under review

booking confirmation

Verified notice that an appointment has been scheduled.

IN USE obtain booking confirmation

appointment availability

Times or capacity potentially available for booking.

IN USE check appointment availability

referral coordinator

Person organizing referral tracking and related communication.

IN USE name the referral coordinator

follow-up checkpoint

Agreed point for checking progress or reporting status.

IN USE set a follow-up checkpoint

Precision check

Sent, received, reviewed, offered, booked, attended, and reported are different stages. Even referral complete can have different system meanings. State the actual event instead of assuming a status label proves that the patient has been seen.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

dependency

Event or requirement that another step relies on.

IN USE identify a booking dependency

supporting documentation

Information accompanying a request to support its assessment.

IN USE supply supporting documentation

referral criteria

Requirements used by a service to assess an appropriate referral.

IN USE clarify referral criteria

clinical priority

Urgency or relative need determined through clinical assessment.

IN USE refer clinical-priority questions

waiting list

Record of people awaiting an available service.

IN USE verify waiting-list status

cancellation slot

Appointment time made available when an existing booking is canceled.

IN USE check a cancellation slot

appointment offer

Proposed date or time not necessarily accepted or finalized.

IN USE confirm an appointment offer

attendance

Actual presence at an appointment or service.

IN USE confirm attendance

referral completion

Completion of the intended referral process, with its definition made clear.

IN USE document referral completion

consultation report

Account returned after a specialist or other consultation.

IN USE obtain the consultation report

care coordination

Organization of care activities and information across people or services.

IN USE support care coordination

access barrier

Practical obstacle to obtaining an intended service.

IN USE identify an access barrier

Precision check

A Friday status check is not a Friday appointment or a clinical recommendation to wait until Friday. Keep administrative tracking separate from urgent clinical needs. A named coordinator improves follow-up without controlling the specialist decision or appointment availability.

Phrases you can actually use

C Notice the language / 1 of 2

IDENTIFY THE REFERRAL

"We are discussing referral F8."

STATE RECEIPT

"The specialist office received it Tuesday."

CORRECT THE APPOINTMENT INFERENCE

"No appointment has been booked."

NAME THE OWNER

"Lia has agreed to check the status."

STATE TRANSMISSION

"It was sent Monday."

EXPLAIN THE STAGE

"They report that it is under review."

DEFINE FRIDAY

"Friday is Lia's status check, not your appointment."

AVOID A REVIEW GUARANTEE

"We do not have a confirmed review-completion date."

They report

Attributes the status to the receiving office.

No appointment

Explicitly corrects the patient's central misunderstanding.

Not your appointment

Prevents a follow-up day from becoming an arrival instruction.

Phrases you can actually use

C Notice the language / 2 of 2

KEEP AN OFFER DISTINCT

"An offered time still needs the relevant booking confirmation."

PRESERVE THE NEXT STEP

"Lia will check what stage the referral has reached."

CLARIFY ACCESS NEEDS

"We can communicate relevant access needs through the proper process."

CHECK UNDERSTANDING

"The referral is received and under review, with no booked appointment."

DO NOT INVENT A CAUSE

"I do not have a verified reason for the review duration."

ROUTE CLINICAL CONCERN

"A new or urgent concern needs our clinical route."

AVOID A COVERAGE CLAIM

"Referral receipt does not itself confirm insurance coverage."

CLOSE PRECISELY

"The confirmed next action is Lia's Friday status check."

Has agreed

Identifies a real owner rather than a vague someone.

Confirmed next action

Offers certainty about the known task without controlling external decisions.

New or urgent concern

Keeps clinical needs separate from administrative timing.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which update accurately uses all three days?

- A** Review must finish Friday because Lia will call.
- B** Sent Monday, received Tuesday, and Lia checks status Friday; no appointment is booked.
- C** Booked Monday, attended Tuesday, and results Friday.
- D** Received Tuesday, so Friday attendance is confirmed.

2. Which sentence creates a false expectation?

- A** Lia will check the status Friday.
- B** The referral is under review.
- C** No appointment is booked.
- D** We will see you at the specialist on Friday.

3. What does receipt establish?

- A** Insurance coverage is guaranteed.
- B** The patient attended.
- C** The referral reached the receiving office, not that a slot is booked.
- D** The specialist completed an assessment.

4. How should a new clinical concern be handled?

- A** By treating the referral identifier as an urgency score
- B** Through the clinic's appropriate clinical route rather than waiting for Lia's routine check
- C** By assuming Friday is always safe
- D** By inventing a specialist assessment over reception

Answers and explanations: [page 93](#).

Friday is a status check

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

booking confirmation / Under review / appointment offer / clinical priority / follow-up checkpoint / receiving service / receipt acknowledgment / referral coordinator / referral identifier / care coordination

- 01 **Morgan:** I heard the specialist received my referral on Tuesday, and someone mentioned Friday. What time should I arrive for the appointment?
- 02 **Omar:** Let me clarify the ¹ _____. Friday is when Lia will check its status. No specialist appointment has been booked.
- 03 **Morgan:** I thought received meant they had accepted me into a slot. Is there still another step before I can arrange time away from work?
- 04 **Omar:** The ² _____ means the office received the referral. They say it is under review, and we do not yet have a date for you to attend.
- 05 **Morgan:** Let me get this straight. It went out Monday and reached them Tuesday. What exactly is Lia doing on Friday if I am not booked in?
- 06 **Omar:** The ³ _____ is F8. It was sent Monday, received Tuesday, and Lia has agreed to check the status Friday.
- 07 **Morgan:** Does under review mean the referral has been rejected, or that the office is still deciding what happens next?
- 08 **Omar:** ⁴ _____ is the status they gave us. We should not turn it into either a rejection or an appointment confirmation without further information.
- 09 **Morgan:** Who is Lia in this process? I need to know whether she is the person checking the status or the clinician deciding about my care.
- 10 **Omar:** Lia is the ⁵ _____ handling this follow-up. Her Friday check does not control the specialist's decision or guarantee that their review will finish that day.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

- 11 **Morgan:** I nearly asked for Friday off work. Please make the next message clear about whether it is an update or an actual appointment offer.
- 12 **Omar:** Correct. A ⁶ _____ is different from the status check. At present, no appointment date or time is confirmed.
- 13 **Morgan:** If they offer a time later, I may need to explain my work schedule and transport arrangements. I cannot assume every offered slot is possible.
- 14 **Omar:** An ⁷ _____ should be handled through the booking process. Relevant access needs can be communicated appropriately rather than assuming an offered time has already been accepted.
- 15 **Morgan:** I also wondered whether the referral being received means my plan has approved the visit. I do not want to confuse the two processes.
- 16 **Omar:** The ⁸ _____ receiving a referral does not itself confirm coverage. Any coverage question needs the appropriate verification, separate from the current referral status.
- 17 **Morgan:** What if the concern I was referred for changes before Friday? Should I simply wait because that is when Lia has promised to check?
- 18 **Omar:** No administrative checkpoint establishes that waiting is safe. A change or urgent concern needs our clinical route for ⁹ _____, not an assumption based on Lia's diary.
- 19 **Morgan:** Understood. The referral reached them Tuesday and remains under review. Lia will check Friday, but I do not have an appointment yet.
- 20 **Omar:** Exactly. That is the current ¹⁰ _____ position. I will keep the status and ownership clear, and any actual booking information must be confirmed separately.

ANSWERS / EXPLANATIONS: PAGE 94

1. follow-up checkpoint | 2. receipt acknowledgment | 3. referral identifier | 4. Under review | 5. referral coordinator | 6. booking confirmation | 7. appointment offer | 8. receiving service | 9. clinical priority | 10. care coordination

Separate another checkpoint from booking

F Say it

Referral G3 was received Wednesday and remains under review. No appointment is booked. Coordinator Noor will check Monday.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

G3 / under review / Monday / Wednesday

Assistant: "The referral identifier is ¹ _____."

Patient: "Receipt was confirmed ² _____."

Assistant: "The status remains ³ _____."

Patient: "Noor will check on ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. G3 | 2. Wednesday | 3. under review | 4. Monday

Rehearse with a fixed script

1. Read Morgan and Omar's checked dialogue. Stress sent Monday, received Tuesday, and status check Friday.
2. Switch roles. Repeat the work-schedule question and explain the actual referral stage without inventing a booking.
3. Complete the transfer using the printed referral facts, check the key, and read the checkpoint as an update, not an appointment.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Clarify before leaving

A Read the situation

Support a patient request for a private explanation while preserving the original clinical instruction.

Patient Jules has not yet left the clinic and does not understand a phrase in the after-visit summary. Jules wants an explanation away from reception before leaving. Dr Chen is available. Assistant Mina cannot reinterpret or change the clinical instruction. No exact instruction, treatment detail, or revised plan is supplied. Mina must identify the document and unclear phrase, pass the question to Dr Chen, and arrange the appropriate private conversation. A patient saying yes or receiving the document does not by itself establish understanding.

PEOPLE / Jules: Patient | Mina: Medical assistant

MAKE A QUESTION WELCOME

Patients may hesitate to ask again when staff appear busy. Thank the person for identifying the unclear wording, and avoid treating clarification as failure. A request for privacy can be handled without announcing the personal question across reception.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which document contains the unclear phrase?

- A A new prescription invented by Mina
- C A supplied insurance appeal

- B The after-visit summary
- D A courier manifest

2. Who is available to explain the clinical instruction?

- A Another patient in reception
- C Dr Chen

- B An automated guess
- D Mina acting outside the stated role

3. What does Jules request?

- A A silent change to the instruction
- C A claim that understanding is already confirmed

- B A public discussion across the waiting area
- D An explanation before leaving, away from reception

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

after-visit summary

Document summarizing relevant information and instructions after a clinical encounter.

IN USE review the after-visit summary

clinical instruction

Direction concerning care or treatment from an authorized professional.

IN USE clarify a clinical instruction

plain language

Clear wording suited to the intended audience.

IN USE explain in plain language

technical term

Specialized word used within a professional field.

IN USE explain a technical term

jargon

Specialized expressions that may be unfamiliar outside a group.

IN USE clarify unfamiliar jargon

abbreviation

Shortened form of a word or phrase.

IN USE expand an abbreviation

ambiguity

Wording that permits more than one interpretation.

IN USE resolve an ambiguity

clarification

Explanation that makes meaning more definite or understandable.

IN USE request clarification

reinterpretation

Assigning a meaning that changes or newly determines the original instruction.

IN USE avoid unauthorized reinterpretation

teach-back

Checking an explanation by inviting the person to express their understanding.

IN USE use teach-back appropriately

show-me method

Checking understanding by asking the person to demonstrate an instructed action.

IN USE use the show-me method

chunk and check

Giving information in short portions and checking understanding between them.

IN USE use chunk and check

Precision check

Clarifying where a phrase appears is different from deciding what the clinical instruction means. Mina can preserve the question and connect Jules with Dr Chen. This does not authorize a paraphrase that changes treatment, timing, or conditions.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

health literacy

Ability to find, understand, and use health information and services.

IN USE support health literacy

language access

Appropriate support enabling communication across language differences.

IN USE arrange language access

qualified interpreter

Person with the relevant skills and qualifications to interpret accurately in context.

IN USE request a qualified interpreter

translation

Rendering written information into another language.

IN USE obtain an appropriate translation

interpretation

Rendering spoken or signed communication into another language, or explaining meaning in another context.

IN USE clarify the required interpretation

accessible format

Presentation adapted to a person's communication or access needs.

IN USE provide an accessible format

private discussion

Conversation conducted with appropriate protection from unnecessary hearing.

IN USE arrange a private discussion

patient question

Specific issue the person wants clarified.

IN USE preserve the patient question

understanding check

Method used to establish whether an explanation was understood.

IN USE conduct an understanding check

instruction source

Document or professional from which a direction came.

IN USE identify the instruction source

revised plan

Updated clinical plan established by the appropriate professional.

IN USE confirm a revised plan

question handoff

Transfer of a specific question to the person who can address it.

IN USE complete a question handoff

Precision check

Teach-back checks the explanation, not the intelligence of the patient. A yes answer or a handed-over summary does not establish comprehension. In this scene, Dr Chen must explain the clinical wording; no new medical direction should be invented during practice.

Phrases you can actually use

C Notice the language / 1 of 2

WELCOME THE QUESTION

"Thank you for telling me which part is unclear."

PRESERVE THE WORDING

"I will keep the original wording with your question."

NAME THE AVAILABLE CLINICIAN

"Dr Chen is available to explain it."

RESPECT PRIVACY

"We can arrange the appropriate conversation away from reception."

IDENTIFY THE DOCUMENT

"The phrase is in your after-visit summary."

STATE THE BOUNDARY

"I cannot reinterpret this clinical instruction."

RESPECT THE TIMING

"You would like clarification before you leave."

AVOID A GUESSED MEANING

"I do not want to give you an explanation that changes the instruction."

06

Which part

Makes the request specific without requiring a public account of personal details.

Keep the original

Protects the instruction from accidental alteration during a handoff.

Available to explain

Names a real route instead of leaving the patient with a refusal.

Phrases you can actually use

C Notice the language / 2 of 2

MAKE THE HANDOFF SPECIFIC

"Dr Chen, Jules has a question about this phrase in the summary."

INVITE A CLEARER EXPLANATION

"Please ask Dr Chen to explain the phrase in language that works for you."

DISTINGUISH SUPPORT NEEDS

"An interpreter request and a clinical meaning question may require different support."

KEEP CHANGES ATTRIBUTED

"Any revised clinical plan must come from the authorized clinician."

DO NOT IMPLY UNDERSTANDING

"Receiving the summary does not mean every instruction is clear."

KEEP THE QUESTION INTACT

"I will not reduce your concern to a generic request for information."

AVOID SHAMING

"Asking again is a reasonable part of understanding the plan."

CLOSE THE ROUTE

"I will connect the question with Dr Chen before you leave."

06

Before you leave

Preserves the patient's timing request.

Language that works for you

Invites understandable communication without blaming the listener.

Any revised plan

Distinguishes explaining an existing direction from changing it.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which response best preserves Jules's request?

- A** Dr Chen is available; I will pass on this phrase and arrange the appropriate private explanation before you leave.
- B** I will guess what the instruction probably means.
- C** You received the paper, so clarification is complete.
- D** Ask another patient what they did.

2. What does handing over the summary establish?

- A** Teach-back was completed.
- B** A revised plan was approved.
- C** Every abbreviation was explained.
- D** The document was provided, not that the patient understands every instruction.

3. Which statement misuses teach-back?

- A** It may require another explanation.
- B** It is a test to prove the patient is intelligent enough.
- C** It helps check whether an explanation was clear.
- D** It can reveal a misunderstanding.

4. What must remain unchanged in Mina's handoff?

- A** A new dose invented for simplicity
- B** An unsupported promise of treatment success
- C** The original instruction and the substance of the patient's question
- D** A guessed clinical meaning supplied by Mina

Answers and explanations: page 95.

Clarify before leaving

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

revised plan / clinical instruction / teach-back / plain language / instruction source / patient question / question handoff / after-visit summary / private discussion / technical term

06

- 01 **Jules:** Mina, I have not left yet because one phrase on this paper is unclear. I can read the words, but I do not know what I am meant to do.
- 02 **Mina:** Thank you for raising it. This is your ¹ _____. Please indicate the phrase, and I will pass the actual wording and your question to Dr Chen.
- 03 **Jules:** Could you just explain what it means? I know everyone is busy, but I do not want to go home and make the wrong assumption.
- 04 **Mina:** I cannot reinterpret the ² _____. Dr Chen is available, so I can connect you with the clinician who can explain it properly.
- 05 **Jules:** I would prefer not to discuss the personal details across reception. There are several people waiting, and this is not something I want them to hear.
- 06 **Mina:** We can arrange an appropriate ³ _____. I will tell Dr Chen that you want clarification before leaving and that the setting matters to you.
- 07 **Jules:** Thank you. I sometimes nod because I recognize a term, then realize I do not understand how it applies to the instruction.
- 08 **Mina:** Recognizing a ⁴ _____ does not always make the instruction clear. Asking again is reasonable; receiving the paper does not mean every question has been answered.
- 09 **Jules:** Could you show Dr Chen this line on the summary? It is this phrase, not the whole page, that I cannot make sense of.
- 10 **Mina:** I will preserve the ⁵ _____ with the wording. That gives Dr Chen the actual issue rather than a vague message that could miss it.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Jules:** I would like a plain explanation, not just the same phrase repeated more loudly. I heard it; the meaning is what I need.
- 12 **Mina:** You can request ⁶ _____ from Dr Chen. I will not substitute my own clinical interpretation while arranging that explanation.
- 13 **Jules:** After Dr Chen explains it, can I say back what I would actually do? I sometimes say yes too quickly when someone asks whether I understand.
- 14 **Mina:** Yes. That kind of ⁷ _____ can help the clinician identify what still needs clarification. It is not a test of your intelligence or a reason to feel embarrassed.
- 15 **Jules:** I also want the original wording available during the conversation. Otherwise someone might explain a shortened version and miss the part that confused me.
- 16 **Mina:** Keeping the ⁸ _____ with the question will help. We should not edit the direction simply to make the handoff shorter.
- 17 **Jules:** So there is no new instruction from this conversation yet. The next step is for Dr Chen to clarify the existing phrase privately before I leave.
- 18 **Mina:** Correct. No ⁹ _____ has been established here. I am arranging the explanation, not changing the clinical direction.
- 19 **Jules:** Please tell Dr Chen that I am still here and that I want to understand the instruction before going. I appreciate having a clear next step.
- 20 **Mina:** I will make that ¹⁰ _____ now: the phrase in your summary, your request for privacy, and clarification before leaving. Dr Chen is the available clinician for that conversation.

06

ANSWERS / EXPLANATIONS: PAGE 96

1. after-visit summary | 2. clinical instruction | 3. private discussion | 4. technical term | 5. patient question | 6. plain language | 7. teach-back | 8. instruction source | 9. revised plan | 10. question handoff

Route an unclear abbreviation

F Say it

Patient Ari asks about an abbreviation in the visit summary before leaving. Assistant Lee cannot interpret the clinical instruction. Dr Malik is available for a private explanation.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

private / Malik / leaving / abbreviation

Lee: "The unclear item is an ¹ _____."

Ari: "I want clarification before ² _____."

Lee: "Dr ³ _____ is available."

Ari: "I would like the explanation to be ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. abbreviation | 2. leaving | 3. Malik | 4. private

Rehearse with a fixed script

1. Read Jules and Mina's corrected exchange. Keep the unclear phrase, original summary, and private explanation connected.
2. Switch roles. Repeat the request to explain understanding in the patient's own words without inventing the clinical instruction.
3. Complete Ari's abbreviation exchange, check the key, then read it aloud with Dr Malik and before leaving clearly stated.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

After the clinical connection

A Read the situation

Confirm a received clinical handoff and record its actual outcome without inventing an assessment or delaying care.

Caller Ellis asked assistant Noor whether a new concern could wait until next week's routine appointment. Noor could not assess urgency and immediately connected Ellis with nurse Dana through the clinic process. Dana received the call. This later conversation reviews the handoff record; it does not delay the live clinical call. No symptom details, clinical assessment, or waiting advice are supplied. Noor must record the actual recipient and outcome without inventing a clinical conclusion. Applicable emergency procedures take priority over routine call handling or documentation.

PEOPLE / Noor: Medical assistant | Dana: Receiving nurse

07

A CALM CALLER CAN STILL NEED ASSESSMENT

Tone of voice is not a reliable substitute for clinical information. Be respectful and direct when someone asks for reassurance you cannot provide. Name the available clinical route immediately, rather than giving a long explanation of your limitations.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is Ellis asking Noor to decide?

A A completed medication review

B Whether a new concern can safely wait until next week's visit

C The spelling of a street name

D A confirmed test result

2. Who is available now?

A Nurse Dana through the clinical-call route

B An unspecified clinician next month

C A specialist appointment already booked

D No one in the clinic

3. What does the lack of supplied symptom details establish?

A The concern is harmless.

B The caller has no symptoms.

C A routine visit is definitely enough.

D The exercise has not established urgency or safety to wait.

[Answers and explanations: page 97.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

clinical-call route

Established pathway for calls requiring clinical review.

IN USE use the clinical-call route

triage

Clinical assessment that determines urgency and appropriate next care.

IN USE refer for triage

urgency assessment

Evaluation of how promptly a concern requires attention.

IN USE obtain an urgency assessment

routine appointment

Scheduled visit not itself proof that a new concern can wait.

IN USE distinguish a routine appointment

new concern

Newly raised issue requiring appropriate handling.

IN USE report a new concern

clinical escalation

Moving an issue to the appropriate level of clinical attention.

IN USE initiate clinical escalation

warm transfer

Call transfer with an introduction or relevant handoff to the receiving person.

IN USE arrange a warm transfer

blind transfer

Transfer without a direct introduction or confirmed contextual handoff.

IN USE avoid an unsupported blind transfer

receiving clinician

Clinical professional accepting the concern or handoff.

IN USE identify the receiving clinician

callback queue

List of requests awaiting a return call.

IN USE check the callback queue

callback commitment

Agreed promise about a return contact, with scope made clear.

IN USE confirm a callback commitment

call disposition

Recorded outcome or next handling status of a call.

IN USE document the call disposition

Precision check

A routine appointment is an administrative booking, not a clinical assessment of a new concern. Calm speech, a familiar patient, or an empty symptom field cannot establish that waiting is safe. Follow the actual clinical and emergency pathways.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

dropped call

Telephone connection that ends unexpectedly.

IN USE follow the dropped-call process

verified callback number

Return-contact number checked through the appropriate procedure.

IN USE obtain a verified callback number

identity check

Process of confirming who is communicating.

IN USE complete the identity check

emergency pathway

Established route for immediate emergency response.

IN USE follow the emergency pathway

red flag

Finding or feature that may signal a serious concern requiring appropriate action.

IN USE escalate a reported red flag

safety-net advice

Professional guidance about changes or circumstances requiring further help.

IN USE provide authorized safety-net advice

symptom onset

Time a reported symptom began.

IN USE relay symptom onset

symptom progression

How a symptom has changed over time.

IN USE describe symptom progression

reassurance

Statement intended to reduce concern, which must not exceed evidence or authority.

IN USE avoid unsupported reassurance

closed-loop communication

Exchange that confirms receipt and intended action.

IN USE use closed-loop communication

SBAR

Situation, background, assessment, recommendation: a structured communication framework.

IN USE use SBAR within the actual role

escalation outcome

Result of routing a concern, including whether it was received.

IN USE record the escalation outcome

Precision check

A queued message is not a connected clinical call. Preserve the actual outcome and follow the clinic process if transfer fails or the connection drops. A structured handoff must not contain an assessment or recommendation invented by someone outside the authorized role.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE QUESTION

"You want to know whether this can wait until next week."

OFFER THE IMMEDIATE ROUTE

"Nurse Dana is available through our clinical-call route now."

SEPARATE THE BOOKING

"The routine appointment does not answer the new clinical question."

USE THE ACTUAL PATHWAY

"We need to follow the clinic's clinical-call process."

STATE THE LIMIT BRIEFLY

"I cannot assess urgency or tell you it is safe to wait."

AVOID UNSUPPORTED REASSURANCE

"I do not have a clinical assessment that would support that reassurance."

PRESERVE THE REQUEST

"I will pass on your question about how soon you need care."

CONFIRM THE RECIPIENT

"I will identify the person receiving the concern."

07

You want to know

Shows understanding of the actual question before routing it.

Cannot assess urgency

Names the specific boundary instead of a vague refusal to help.

Available now

Makes the next step concrete and timely.

Phrases you can actually use

C Notice the language / 2 of 2

PROTECT CONTINUITY

"We should use the verified contact process if the connection fails."

KEEP FACTS ATTRIBUTED

"I will relay what you report without adding a diagnosis."

KEEP EMERGENCY PRIORITY

"Applicable emergency procedures take precedence over routine call handling."

RECORD THE OUTCOME

"The note must show whether the clinical transfer succeeded."

DO NOT CLAIM CONNECTION EARLY

"A message in a queue is not the same as a completed transfer."

AVOID GUESSED ADVICE

"I will not invent treatment or safety-net instructions."

CHECK RECEIPT

"Please confirm that the receiving clinician has the concern."

CLOSE THE HANDOFF

"The timing question belongs with the available clinical team now."

Does not answer

Separates the scheduled visit from the new concern.

Whether it succeeded

Keeps the record focused on outcome rather than effort.

Without adding

Preserves the caller's information and avoids invented clinical content.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which response addresses the caller's need appropriately?

- A** If you sound calm, next week is fine.
- B** A routine appointment always means the concern can wait.
- C** No details were supplied, so nothing is wrong.
- D** I cannot assess whether waiting is safe; Dana is available now through our clinical-call route.

2. What does an unanswered transfer establish?

- A** The issue is resolved.
- B** The clinical connection has not been completed.
- C** Dana received and assessed the concern.
- D** The caller was reassured by a clinician.

3. Which information must not be invented for a structured handoff?

- A** A diagnosis or urgency decision outside Noor's role
- B** The caller's actual question
- C** The receiving person's name when confirmed
- D** The actual transfer outcome

4. What takes precedence when applicable?

- A** Completing an English practice dialogue
- B** Obtaining a routine administrative update first
- C** The actual emergency pathway
- D** Waiting for next week's booking regardless of circumstances

Answers and explanations: [page 97](#).

After the clinical connection

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

urgency assessment / escalation outcome / emergency pathway / routine appointment / receiving clinician / clinical-call route / callback queue / triage / warm transfer / dropped call

07

- 01 **Noor:** Dana, thanks for taking Ellis's call immediately. I am checking my handoff note now, after the connection. Ellis asked whether a new concern could wait until next week.
- 02 **Dana:** I received that question. Your note should not contain an ¹ _____ from you. Receiving the call and assessing the concern are different events.
- 03 **Noor:** I told Ellis I could not decide that and connected the call through our process. I did not ask Ellis to finish a long administrative discussion first.
- 04 **Dana:** Good. The request needed ² _____, not a yes-or-no answer from reception. Keep the actual clinical response in the appropriate clinical record.
- 05 **Noor:** Ellis already had a booking next week. I included that as background, but I did not say the booking established a safe waiting period.
- 06 **Dana:** Correct. A ³ _____ does not assess a new concern. Do not let the appointment date become the only information the next person receives.
- 07 **Noor:** My note currently says passed to nursing. That leaves out who received the call. I should name you and record the actual connection.
- 08 **Dana:** Yes. The ⁴ _____ worked in this case. Record that factual outcome, not simply an attempt to put somebody through.
- 09 **Noor:** I gave you Ellis's question without describing Ellis as anxious or suggesting the concern was mild. Neither would have been a finding I had established.
- 10 **Dana:** As the ⁵ _____, I need the actual report. Your introduction helped me understand why Ellis was calling rather than making Ellis start with the appointment history again.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Noor:** I stayed for the introduction and confirmed the connection before leaving the call. I will describe what happened rather than just checking a transfer box.
- 12 **Dana:** That is the relevant ⁶ _____ detail. Keep the actual handoff time in the record; do not substitute the later time at which you finish writing.
- 13 **Noor:** For a future call, if the line disconnects before anyone confirms receipt, I cannot use today's completed status as the default.
- 14 **Dana:** Exactly. Follow the clinic's ⁷ _____ process. Verified contact details and the actual unresolved concern matter; do not silently leave a failed connection marked complete.
- 15 **Noor:** And if I only put a request into a message queue, the note should still show pending contact, not that the patient spoke to a nurse.
- 16 **Dana:** Right. A ⁸ _____ is not a clinical conversation. Use the appropriate follow-up and escalation process for the real situation, rather than assume the queue resolves urgency.
- 17 **Noor:** This record review happened after the immediate connection. We should never make an active caller wait while we discuss all these documentation examples.
- 18 **Dana:** Agreed. The actual ⁹ _____ takes precedence when needed. Do not use this case to decide that another caller is nonurgent.
- 19 **Noor:** I will record your confirmed receipt and my factual introduction. I will not add a diagnosis, waiting advice, or clinical outcome that you have not provided.
- 20 **Dana:** That keeps the ¹⁰ _____ accurate. The call reached me; your note should say that clearly while leaving the clinical findings to the clinical record.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. urgency assessment | 2. triage | 3. routine appointment | 4. clinical-call route | 5. receiving clinician | 6. warm transfer | 7. dropped call | 8. callback queue | 9. emergency pathway | 10. escalation outcome

Preserve an interrupted clinical transfer

F Say it

A clinical transfer for caller Ari disconnects before receipt is confirmed. Assistant Lee must use the clinic dropped-call process. Nurse Priya is the intended clinical recipient. No assessment has occurred.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

Priya / assessment / confirmed / dropped-call

Lee: "Receipt has not been ¹ _____."

Colleague: "Use the clinic's ² _____ process."

Lee: "The intended recipient is nurse ³ _____."

Colleague: "No clinical ⁴ _____ has occurred."

ANSWERS / EXPLANATIONS: PAGE 98

1. confirmed | 2. dropped-call | 3. Priya | 4. assessment

Rehearse with a fixed script

1. Read Noor and Dana's corrected review. Stress received, not independently assessed, and the actual call outcome.
2. Switch roles. Repeat the distinction between a connected call and a queued message without adding a clinical result.
3. Complete Ari's interrupted-transfer exchange, check the key, then read it aloud without claiming receipt or assessment.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Thursday, not Tuesday

A Read the situation

Correct an appointment message with a clear apology, verified booking details, and an accurate contact record.

Assistant Elena sent patient Robin a message incorrectly saying that a results discussion was booked Tuesday. The verified schedule shows Thursday at 14:00 with Dr Chen. Robin rearranged work after the incorrect message. Elena can correct the communication and confirm the actual booking, but no test results or clinical interpretation are supplied. The appointment itself has not been changed from Tuesday to Thursday; the earlier message was wrong. Elena must preserve that distinction, acknowledge the disruption, and document whether the correction actually reached Robin.

PEOPLE / Robin: Patient | Elena: Medical assistant

08

CORRECT THE MESSAGE WITHOUT REWRITING THE HISTORY

An apology is more useful when it names what went wrong and supplies the verified replacement information. Avoid describing an inaccurate message as a rescheduled visit if the booking never changed. Acknowledge the practical effect without making an unsupported compensation promise.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the verified schedule show?

- A An appointment already attended
- C Thursday at 14:00 with Dr Chen

- B A canceled results discussion
- D Tuesday at 14:00 with a different clinician

2. What caused Robin to rearrange work?

- A A supplied abnormal result
- C A confirmed clinical emergency

- B Elena's inaccurate Tuesday message
- D An actual schedule change from Tuesday

3. Which topic is not supplied for discussion?

- A The effect of the wrong message
- C The verified appointment details

- B The actual test results
- D The correction ownership

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

appointment confirmation

Verified communication of a scheduled visit and its details.

IN USE send an appointment confirmation

incorrect notification

Message containing inaccurate information.

IN USE correct an incorrect notification

schedule of record

Authoritative schedule used to verify the actual booking.

IN USE check the schedule of record

message correction

Communication replacing an inaccurate earlier message.

IN USE issue a message correction

rescheduling

Changing an actual appointment from one slot to another.

IN USE distinguish rescheduling from correction

results discussion

Appointment or conversation in which a clinician explains test findings.

IN USE confirm a results discussion

result interpretation

Clinical explanation of what findings mean in context.

IN USE refer result interpretation

date verification

Checking the actual calendar date associated with a booking.

IN USE complete date verification

twenty-four-hour time

Time notation running from 00:00 through 23:59.

IN USE use twenty-four-hour time

time-zone reference

Identification of the time zone when relevant to communication.

IN USE clarify the time-zone reference

read-back

Repeating important information to confirm accurate receipt.

IN USE request a read-back

correction timestamp

Recorded time at which an amendment or correction occurred.

IN USE preserve the correction timestamp

08

Precision check

Correcting Tuesday to Thursday in a message is not necessarily rescheduling. The verified booking was Thursday; the communication was wrong. Keep that history accurate so a later colleague does not infer a clinical or scheduling decision that never occurred.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

original entry

Earlier record item retained under the applicable process.

IN USE preserve the original entry

addendum

Additional note linked to an existing record.

IN USE enter an addendum

audit trail

Record showing the sequence and authorship of events or changes.

IN USE maintain an audit trail

communication log

Record of relevant contacts and their outcomes.

IN USE update the communication log

contact attempt

Effort to reach someone without assuming successful contact.

IN USE document a contact attempt

successful contact

Confirmed communication with the intended recipient.

IN USE verify successful contact

acknowledgment

Confirmation of receipt or recognition of information.

IN USE record acknowledgment

follow-up owner

Person responsible for a specified unfinished next action.

IN USE name the follow-up owner

outstanding action

Task that remains to be completed.

IN USE track the outstanding action

service disruption

Practical interference caused by a service problem.

IN USE acknowledge the service disruption

escalation route

Defined path for a concern requiring additional attention or authority.

IN USE explain the escalation route

closure evidence

Information establishing that the intended task was actually completed.

IN USE retain closure evidence

08

Precision check

A results discussion appointment does not reveal whether results are normal, abnormal, urgent, or already reviewed. Confirm the booking within the role and route clinical questions appropriately. In real communication, verify the full date and relevant location or time-zone details.

Phrases you can actually use

C Notice the language / 1 of 2

OPEN THE CORRECTION

"I need to correct the appointment message I sent you."

NAME THE ERROR

"I wrote Tuesday, but that was incorrect."

GIVE THE VERIFIED BOOKING

"The schedule confirms Thursday at 14:00 with Dr Chen."

TRANSLATE THE TIME

"Fourteen hundred is two in the afternoon."

08

DISTINGUISH THE EVENT

"This corrects my message; it is not a change from a Tuesday booking."

APOLOGIZE DIRECTLY

"I am sorry that my incorrect message disrupted your work arrangements."

AVOID A RESULT INFERENCE

"The appointment details do not tell us what the results mean."

KEEP THE ROLE CLEAR

"Dr Chen will handle the clinical discussion."

I wrote

Names the source of the error without vague blame.

But that was incorrect

Explicitly withdraws the earlier information.

Not a change

Distinguishes a message repair from a real booking change.

Phrases you can actually use

C Notice the language / 2 of 2

CHECK ACCURATE RECEIPT

"Please read back the verified day and time."

DOCUMENT CONTACT HONESTLY

"An attempted call is not the same as speaking with you."

TRACK UNFINISHED WORK

"Any unsuccessful contact must remain an open action."

AVOID AN UNSUPPORTED PROMISE

"I cannot promise a remedy that has not been authorized."

PRESERVE THE ORIGINAL

"The record should retain the earlier message and its correction appropriately."

NAME THE OWNER

"I am responsible for communicating this correction."

OFFER THE ACTUAL ROUTE

"We can use the clinic's process for any unresolved service concern."

CLOSE WITH THE VERIFIED FACTS

"Thursday at 14:00 with Dr Chen is the confirmed booking."

08

Disrupted your work

Acknowledges the specific effect instead of a generic inconvenience.

Read back

Checks factual receipt of appointment details.

Remain an open action

Prevents an unsuccessful attempt from being marked complete.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which correction preserves the history?

- A** Your appointment has been moved from Tuesday even though the schedule never changed.
- B** Tuesday and Thursday mean the same thing here.
- C** The booking is unverified because a message was wrong.
- D** My Tuesday message was wrong; the verified booking is Thursday at 14:00 with Dr Chen.

2. Which conclusion about results is unsupported?

- A** The booking is for a results discussion.
- B** Thursday means the results are definitely normal.
- C** Dr Chen is the named clinician.
- D** No results are supplied in the exercise.

3. What should be recorded if Robin is not reached?

- A** The contact attempt and the still-outstanding correction
- B** Confirmed patient receipt
- C** Completed results counseling
- D** Robin's agreement to the booking

4. Which apology is strongest?

- A** I am sorry you understood the Tuesday message differently.
- B** I am sorry the schedule changed, although the verified booking never changed.
- C** I am sorry for the inconvenience; the original date does not matter now.
- D** I am sorry my incorrect Tuesday message disrupted your work arrangements.

Answers and explanations: page 99.

Thursday, not Tuesday

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

message correction / results discussion / twenty-four-hour time / follow-up owner / audit trail / result interpretation / rescheduling / schedule of record / successful contact / escalation route

08

- 01 **Robin:** Elena, I arranged time off for the Tuesday results discussion, but the latest information says Thursday. Has the appointment been moved again?
- 02 **Elena:** I need to make a ¹ _____. My Tuesday message was incorrect. The verified schedule shows Thursday at 14:00 with Dr Chen.
- 03 **Robin:** That matters because I changed my work shift after reading what you sent. I need to know whether Thursday is confirmed or just another possible date.
- 04 **Elena:** I have checked the ² _____. Thursday at 14:00 with Dr Chen is confirmed. I am sorry my incorrect message disrupted your work arrangements.
- 05 **Robin:** I have the Tuesday message right here. Was that date wrong from the start, or did someone move my appointment without telling me?
- 06 **Elena:** You are right. This is not ³ _____. The earlier message was wrong; I should not describe that as moving an appointment that was booked for Tuesday.
- 07 **Robin:** Fourteen hundred means two in the afternoon, correct? I want to check the time as well as the day before I speak to my manager.
- 08 **Elena:** Correct. That is ⁴ _____ for two in the afternoon. Please keep Thursday, 14:00, and Dr Chen together when you read back the details.
- 09 **Robin:** Thursday at two in the afternoon with Dr Chen. Does the later day tell me anything about whether the results are normal or serious?
- 10 **Elena:** No. A ⁵ _____ booking does not tell us what the results mean. I can confirm the appointment, but the clinical explanation belongs with Dr Chen.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Robin:** I understand. Please do not let my question about the date become a note saying that someone already explained the results to me.
- 12 **Elena:** I will keep ⁶ _____ separate from this correction. The record should describe the appointment information communicated, not a clinical conversation that did not happen.
- 13 **Robin:** Will the earlier message remain visible appropriately? Otherwise a later colleague may think I simply chose the wrong day when I arranged work.
- 14 **Elena:** We need the appropriate ⁷ _____. I will preserve the earlier event and the correction through our process rather than erase the reason for your confusion.
- 15 **Robin:** And will you record that you actually reached me today? I would like the team to know I received and repeated back the Thursday information.
- 16 **Elena:** Yes. This is ⁸ _____, not merely an attempted call. I will document the actual communication and acknowledgment through the required process.
- 17 **Robin:** Please send the corrected details through the clinic's usual channel too. I need something accurate to refer to when I rearrange my shift.
- 18 **Elena:** Exactly. A named ⁹ _____ would need to keep that action visible. I own this correction and will record what has actually been communicated.
- 19 **Robin:** Thank you for acknowledging the work disruption. I may still raise that concern through the clinic process, but I now have the confirmed appointment details.
- 20 **Elena:** We can use the appropriate ¹⁰ _____ for that concern. The verified booking remains Thursday at 14:00 with Dr Chen, and I am sorry for the inaccurate Tuesday message.

08

ANSWERS / EXPLANATIONS: PAGE 100

1. message correction | 2. schedule of record | 3. rescheduling | 4. twenty-four-hour time | 5. results discussion | 6. result interpretation | 7. audit trail | 8. successful contact | 9. follow-up owner | 10. escalation route

Correct a different time error

F Say it

Assistant Noor sent 09:00 in error. The verified booking is Friday at 11:30 with Dr Shah. The appointment itself has not changed. Patient Ari has now received the correction.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

09:00 / Shah / rescheduling / 11:30

08

Noor: "My message incorrectly said ¹ _____."

Ari: "The verified time is ² _____."

Noor: "The clinician is Dr ³ _____."

Ari: "This is a message correction, not ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. 09:00 | 2. 11:30 | 3. Shah | 4. rescheduling

Rehearse with a fixed script

1. Read Robin and Elena's corrected dialogue. Contrast the wrong Tuesday message with the verified Thursday booking.
2. Switch roles. Repeat the apology and read-back, saying 14:00 and two in the afternoon as the same time.
3. Complete the second correction, check the key, and read the verified time without describing the message error as rescheduling.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Book the right language support

E More workplace conversations / 1-10

Confirm spoken and written language preferences separately and distinguish family support from qualified interpreting.

Before tomorrow's visit, assistant Mina and coordinator Jo check Mei's recorded preferences: spoken Mandarin, written Traditional Chinese. Mei requests her sister's support. The clinic has booked a qualified video interpreter for Mandarin; a copied appointment note incorrectly says Cantonese. No diagnosis or treatment discussion is taking place here.

PEOPLE / Mina: Medical assistant | Jo: Language-access coordinator

Use each bank entry once, without changing its form. Answers follow turn 20.

09

WORD + PHRASE BANK

teach-back / qualified interpreter / written-language preference / spoken-language preference / short segments / video interpreting

- 01 **Mina:** Jo, the interpreter booking says Mandarin, but tomorrow's appointment note says Cantonese. Both are attached to Mei's visit. Which preference did we actually confirm with her?
- 02 **Jo:** The recorded ¹ _____ is Mandarin. The Cantonese entry was copied incorrectly. Please correct that entry through our record process and preserve the reason.
- 03 **Mina:** Her written preference is Traditional Chinese. I should keep that as a separate field, rather than assume that the spoken language tells me which written materials she wants.
- 04 **Jo:** Exactly. The ² _____ guides the material request. Check that the available document is the approved version in that form, not just any document labeled Chinese.
- 05 **Mina:** Mei wants her sister in the visit. Should I cancel the interpreter and put family interpreting instead?
- 06 **Jo:** No. Keep our ³ _____ booking. Mei asked for her sister's support, not for us to withdraw the language service.
- 07 **Mina:** I will tell the team that distinction. We should address Mei directly, rather than asking her sister to answer every question because it seems quicker.
- 08 **Jo:** Yes. The clinician should speak to Mei and allow the interpreter to relay the exchange. The sister can contribute within the arrangement Mei agrees to; she does not automatically speak for her.
- 09 **Mina:** The service is by video. I can check the room equipment and booking connection before the visit, without opening a clinical conversation in a public area.
- 10 **Jo:** Please do. Record that it is ⁴ _____, including the appropriate service details. A booking confirmation does not prove the interpreter has joined tomorrow's session.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Mina:** For the handoff, I will say Mandarin interpreter booked and Traditional Chinese materials requested. I will not say all language needs are complete before the document and connection checks.
- 12 **Jo:** Good. If the required document is unavailable, bring that back to me. Do not substitute an unreviewed automatic translation for a clinical instruction and mark it approved.
- 13 **Mina:** The clinician asked whether the interpreter could simply summarize a long explanation at the end. Would shorter sections make the exchange easier to check?
- 14 **Jo:** Yes. Use ⁵ _____ and pause for interpretation. Preserve the patient's questions too, not just the clinician's instructions.
- 15 **Mina:** Mei may want to discuss something without her sister present. We need to offer that opportunity respectfully, without treating the request as a family disagreement.
- 16 **Jo:** Correct. Follow our private-conversation process and ask Mei about her wishes. The interpreter's confidentiality obligations remain important whether or not a family member is present.
- 17 **Mina:** For checking an explanation, the clinician can ask Mei to describe the plan in her own words through the interpreter, rather than just asking whether she understands.
- 18 **Jo:** That is ⁶ _____. The interpreter relays the response; the clinician resolves any clinical misunderstanding.
- 19 **Mina:** I will correct Cantonese to the confirmed Mandarin preference, keep the written preference separate, retain the interpreter booking, and check the materials and video arrangements.
- 20 **Jo:** Thank you. I will check the approved document request. We will leave the remaining preparation tasks visible rather than mark the visit language support complete prematurely.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. spoken-language preference | 2. written-language preference | 3. qualified interpreter | 4. video interpreting | 5. short segments | 6. teach-back

Two preferences, two arrangements

F Say it in a new situation

Luis confirms spoken Spanish and written English. His adult son will attend for support. The clinic has booked a qualified Spanish telephone interpreter. The connection has not yet occurred; approved English paperwork is available.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

telephone / support / English / Spanish

09

Assistant: Luis's spoken preference is ¹ _____.

Coordinator: His written preference is ² _____.

Assistant: The booked interpreter will join by ³ _____.

Coordinator: The son's attendance is for ⁴ _____.

ANSWERS / EXPLANATIONS: PAGE 101

1. Spanish | 2. English | 3. telephone | 4. support

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

A number needs its unit

E More workplace conversations / 1-10

Read back a measurement with its unit and explain a traceable correction without inventing a new measurement or clinical conclusion.

Fictional record check: today's saved scale output is 160.0 lb at 09:12. A draft entry incorrectly says 160.0 kg. Height was recorded separately as 170 cm. Use 1 lb = 0.45359237 kg; round the converted weight to one decimal place. Nurse Dana reviews the correction; clinical use of the draft is not yet checked.

PEOPLE / Rosa: Medical assistant | Dana: Nurse

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

measurement time / 72.6 / unit conversion / source measurement / height / audit trail

10

- 01 **Rosa:** Dana, I found a mismatch before closing the rooming record. The saved scale output says one hundred sixty point zero pounds, but the draft says kilograms.
- 02 **Dana:** Check the ¹ _____, including its unit and time. Do not decide which label looks right from the patient's appearance or a remembered earlier weight.
- 03 **Rosa:** The saved output is clear: 160.0 lb at nine twelve today. If our field requires kilograms, multiplying by the supplied factor gives seventy-two point five seven four eight.
- 04 **Dana:** To one decimal place, enter ² _____ kilograms through the correction process. Keep the original pounds reading and the conversion identifiable; those are not two separate weighings.
- 05 **Rosa:** So changing the unit label alone would be wrong. One hundred sixty kilograms is not another way of expressing one hundred sixty pounds.
- 06 **Dana:** Correct. A ³ _____ preserves the quantity by changing both number and unit appropriately. Also check whether the system already converts values, so the same reading is not converted twice.
- 07 **Rosa:** The height entry is one hundred seventy centimeters. In meters, that is one point seven zero. It is not one hundred seventy inches.
- 08 **Dana:** Right. Keep ⁴ _____ separate from weight. Use the verified value and required unit for each field, not a copied unit from the previous row.
- 09 **Rosa:** Would it help to write that the corrected weight is normal? The number looks more plausible now.
- 10 **Dana:** No. Plausibility is not a clinical assessment. We are correcting a recording error, not assigning a weight category, calculating a treatment dose, or declaring the patient healthy.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Rosa:** I will retain the original entry and record what changed and why under our process. I should not delete the history and make it look as if no error occurred.
- 12 **Dana:** Yes. The ⁵ _____ matters if somebody has already used the draft. Tell me which records or workflows may have received it so we can investigate promptly.
- 13 **Rosa:** I have not checked that yet. I will not say no effect just because the error was in a draft field. You need to know the downstream use is unresolved.
- 14 **Dana:** Thank you. I will handle the clinical implications through the appropriate review. Your job is to supply the measurement evidence and report the discrepancy, not estimate harm from the arithmetic alone.
- 15 **Rosa:** Should the converted entry carry the time I make the correction, or nine twelve when the patient was actually weighed?
- 16 **Dana:** Preserve the ⁶ _____ as nine twelve and record the correction time separately as required. Converting an existing reading does not mean a new measurement occurred.
- 17 **Rosa:** If the scale record had lacked a unit, I would need clarification through our procedure rather than automatically apply the pounds-to-kilograms factor.
- 18 **Dana:** Exactly. This case gives a verified unit. When the source is uncertain, multiplying by a familiar factor can make an unsupported assumption look precise.
- 19 **Rosa:** Read-back: original 160.0 pounds at 09:12; converted 72.6 kilograms to one decimal; separate height 170 centimeters, or 1.70 meters. Correction history retained; downstream use still being checked.
- 20 **Dana:** Confirmed. Bring the corrected record for the required check. The arithmetic is clear, but it does not by itself close the review of where the incorrect entry may have gone.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. source measurement | 2. 72.6 | 3. unit conversion | 4. height | 5. audit trail | 6. measurement time

Convert once and preserve the source

F Say it in a new situation

A verified scale output is 132.0 lb at 10:18. The record requires kilograms to one decimal; use 1 lb = 0.45359237 kg. A separate verified height is 165 cm. The correction is entered at 10:31.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

59.9 / 10:31 / 1.65 / 10:18

Assistant: The weight rounds to ¹ _____ kilograms.

Nurse: The original measurement time remains ² _____.

Assistant: The separate height is ³ _____ meters.

Nurse: The record was corrected at ⁴ _____.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. 59.9 | 2. 10:18 | 3. 1.65 | 4. 10:31

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

A clear trace is not a diagnosis

E More workplace conversations / 1-10

Distinguish ECG electrodes, recorded leads, technical warnings, and clinical interpretation during a clinician handoff.

Assistant Leo is trained and authorized for ECG acquisition under this fictional clinic's procedure, not interpretation. A 12-lead ECG recorded at 11:06 has a lead-off warning. After the prescribed technical checks, an authorized repeat is recorded at 11:10. Its automated wording is unconfirmed. Dr Shah receives both traces and the patient's question.

PEOPLE / Leo: Medical assistant | Dr Shah: Receiving clinician

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

repeat acquisition / reviewing clinician / Automated interpretation / lead-off warning / Signal quality / electrodes

11

- 01 **Leo:** Dr Shah, I have the original eleven-oh-six ECG and the repeat at eleven ten. The first recording showed a lead-off warning. Both are labeled and retained for your review.
- 02 **Dr Shah:** Start with the technical history. A ¹ _____ is not a clinical diagnosis. Tell me what was observed and checked, without deciding what the waveform means.
- 03 **Leo:** I completed the prescribed technical checks within my authorized role, then recorded the approved repeat. I have documented those checks; I have not attributed any clinical finding to a loose connection.
- 04 **Dr Shah:** Good. Keep the acquisition history attached. The patient should not be told everything is normal just because the technical warning no longer appears on the second recording.
- 05 **Leo:** The patient asked why a twelve-lead test uses ten sticky patches. I explained that the patches are electrodes and that lead has another meaning in the test name.
- 06 **Dr Shah:** Yes. The ² _____ make contact with the skin. The twelve leads represent different electrical views, not twelve separate adhesive patches.
- 07 **Leo:** I did not give a placement lesson or change the prescribed positions to improve the picture. Any acquisition difficulty follows our equipment instructions and clinic procedure.
- 08 **Dr Shah:** Exactly. Correct patient identification and recording details matter too. A technically clear trace with the wrong patient label would still be an unsafe record.
- 09 **Leo:** The repeat looks easier to read on screen. May I describe that as better signal quality while leaving the clinical meaning to you?
- 10 **Dr Shah:** Report the actual technical observations. ³ _____ and clinical interpretation answer different questions. Do not turn easier to read into a normal-heart conclusion.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Leo:** The machine also printed an automated statement. I have left it identified as machine-generated and unconfirmed, rather than copying it into a message as your conclusion.
- 12 **Dr Shah:** Correct. ⁴ _____ supports the review but does not replace it. I need the actual recordings and patient context, not just a sentence from the device.
- 13 **Leo:** The patient asked whether the repeat means something serious was found. I said you would explain the findings, and that I repeated the recording for the documented technical reason.
- 14 **Dr Shah:** That keeps the explanation within your role without minimizing the question. Pass any current concern through the clinical route promptly; a technical check must not delay necessary care.
- 15 **Leo:** For the record, both recordings retain their own times. I have not overwritten eleven oh six with eleven ten or made the first trace disappear.
- 16 **Dr Shah:** Thank you. Keep the ⁵ _____ distinguishable from the original. A difference between traces needs review, not an assumption that the patient's condition changed.
- 17 **Leo:** The chart therefore shows the initial warning, documented checks, original and repeat times, and your receipt. It does not say interpreted, normal, or cleared to leave.
- 18 **Dr Shah:** Right. Receipt by the ⁶ _____ is one stage. The clinical findings and next steps belong in the appropriate clinical record after the actual assessment.
- 19 **Leo:** My handoff is complete: both traces, technical history, and the patient's question are with you. No clinical result has been communicated by me.
- 20 **Dr Shah:** Received. I will review the recordings and speak with the patient. Retain your factual acquisition note without adding a result that I have not provided.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. lead-off warning | 2. electrodes | 3. Signal quality | 4. Automated interpretation | 5. repeat acquisition | 6. reviewing clinician

Name the stage that actually occurred

F Say it in a new situation

A standard 12-lead ECG uses 10 skin electrodes. The initial trace is timed 14:02 and an authorized repeat 14:07. Dr Malik has received both, but no clinical interpretation is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

14:02 / 14:07 / supplied / 10

Assistant: This standard twelve-lead recording uses ¹ _____ skin electrodes.

Clinician: The original trace is timed ² _____.

Assistant: The repeat is timed ³ _____.

Clinician: A clinical interpretation is not yet ⁴ _____.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. 10 | 2. 14:02 | 3. 14:07 | 4. supplied

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Clarifying the visit at check-in

A Briefing checks / page 4

1. What does the schedule show?

A. An annual review

The schedule lists an annual review, while the patient's expectation differs.

2. What should Sam do with the name preference?

D. Use Alex while preserving the verified record match.

Alex is the preferred spoken name; that does not justify a duplicate record or an unverified identifier change.

3. Who can clarify the visit plan?

C. Nurse Dana

Dana is the available clinical route, and Sam cannot determine suitability independently.

D Language checks / page 9

1. Which handoff preserves both facts?

A. The schedule says annual review; Alex expected to discuss a recent concern.

The statement preserves the scheduled category and reported expectation without inventing fault or a clinical decision.

2. What does using Alex change here?

B. The respectful form of spoken address, not the verified identity match

The preferred name affects address while record matching and other decisions remain separate.

3. Which reassurance is unsupported?

C. This visit will definitely resolve the concern at no additional cost.

Neither the clinical outcome nor any billing decision is established by the facts.

4. Which action is appropriate?

A. Pass the mismatch to Dana while keeping the correctly matched record.

The available route is clinical clarification with accurate identification, not an unauthorized record or treatment change.

01 Conversation answers

Clarifying the visit at check-in

E Dialogue gaps

Pages 10-11

1. scheduled purpose. Turn 2. The scheduled purpose is the annual review recorded in the booking, which differs from Alex's stated expectation.

2. preferred name. Turn 4. The preferred name is Alex, while the verified record name remains Alexandra and must stay correctly linked.

3. clinical suitability. Turn 6. Clinical suitability requires the appropriate clinical judgment; Sam cannot decide it from the appointment label alone.

4. presenting concern. Turn 8. The presenting concern is what Alex wants addressed, and it should reach the clinical team without unnecessary public disclosure.

5. patient expectation. Turn 10. Patient expectation describes what Alex believed the visit would address, not merely dissatisfaction with an administrative reminder.

6. billing category. Turn 12. A billing category may affect charging, but the scenario supplies no verified charge or coverage decision from the visit label.

7. clinical handoff. Turn 14. The clinical handoff passes the mismatch to Dana, the identified clinical route, without Sam inventing a treatment decision.

8. identity match. Turn 16. The identity match links Alex to the verified existing record and must remain intact while the preferred address is communicated.

9. visit plan. Turn 18. The visit plan concerns what this encounter will address and is the question Dana can clarify with Alex.

10. appointment type. Turn 20. The appointment type remains the recorded annual review, distinct from the clinical clarification now requested.

F Transfer conversation

Page 12

1. follow-up

Lee: "The recorded visit is a follow-up."

Follow-up is the scheduled category stated in this scenario.

2. initial consultation

Morgan: "I expected an initial consultation."

The patient's expectation differs from the recorded follow-up category.

3. Inez

Lee: "Nurse Inez can clarify the plan."

Inez is the available clinical professional named in the facts.

4. established

Lee: "Clinical suitability is not yet established."

No clinical suitability decision has been supplied for this mismatch.

02 Briefing and language checks

Rooming and factual patient reports

A Briefing checks / page 13

1. What onset did Pat report?

C. Monday

Monday is the onset reported by Pat, not an independently measured event.

2. What did Rosa observe?

B. Pat answering questions while seated

Only the seated question-answering observation is supplied; measurements and conclusions are absent.

3. What should remain unassessed in the report?

A. Cause and severity

Rosa has not assessed cause or severity and must not manufacture those findings.

D Language checks / page 18

1. Which report is supported?

B. Pat reports tiredness since Monday; I observed Pat answering questions while seated.

The supported report preserves both supplied sources without adding a cause, severity, or safety judgment.

2. What belongs in the temperature field when no measurement is available?

C. No invented numeric value

No measurement is supplied; neither a previous value nor an impression can establish today's temperature.

3. Why is "Pat is fine" inadequate?

C. It substitutes a broad conclusion for the limited observation.

Fine does not describe the actual observation and implies a clinical reassurance that Rosa has not established.

4. Which statement about other symptoms is accurate?

B. No other symptoms are supplied; that does not prove their absence.

Silence in the case does not constitute a negative clinical history or assessment.

02 Conversation answers

Rooming and factual patient reports

E Dialogue gaps

Pages 19-20

- 1. source attribution.** Turn 2. Source attribution identifies Pat as the source of the symptom report and Rosa as the source of the observation.
- 2. onset.** Turn 4. Onset is the stated beginning of the symptom; Monday is attributed to Pat rather than independently established.
- 3. observation.** Turn 6. The observation is Pat answering questions while seated; it does not establish a diagnosis, severity, or normal measurements.
- 4. Vital signs.** Turn 8. Vital signs require actual measurements and context; expected or typical values cannot fill missing clinical data.
- 5. pertinent negative.** Turn 10. A pertinent negative is an established relevant absence, not a conclusion drawn from an unmentioned symptom.
- 6. patient-reported symptom.** Turn 12. The patient-reported symptom is the experienced tiredness described by Pat; attribution remains necessary even when it is paraphrased.
- 7. baseline.** Turn 14. A baseline supplies the known usual or earlier state needed for a meaningful comparison; it is not supplied here.
- 8. unassessed.** Turn 16. Unassessed means not evaluated through the relevant process, not evaluated and found normal or harmless.
- 9. Clinical assessment.** Turn 18. Clinical assessment is the qualified evaluation still needed; it must not be inferred from Rosa's limited rooming observation.
- 10. severity.** Turn 20. Severity remains unassessed in Rosa's report and cannot be inferred merely from Pat speaking while seated.

F Transfer conversation

Page 21

- 1. dizziness**
Noor: "Lee reports dizziness since breakfast."
Dizziness is the symptom attributed to Lee in the facts.
- 2. breakfast**
Inez: "The reported onset is breakfast."
Breakfast is the reported starting point, not a measured cause.
- 3. seated**
Noor: "I observed Lee speaking while seated."
The supplied observation concerns speaking in a seated position.
- 4. unassessed**
Inez: "The cause remains unassessed."
No assessment of the cause has been supplied here.

03 Briefing and language checks

Medication histories with clear sources

03

A Briefing checks / page 22

1. What does Ren report?

D. Another clinic stopped one medicine.

Ren reports an outside change, but the instruction is not available for verification.

2. What conflicts with that report?

A. The medicine remains on the local list.

The local list still includes the medicine, creating the stated discrepancy.

3. What can Dana do?

D. Review the discrepancy with the clinician

Dana is the identified route for appropriate review, not a supplied treatment decision.

D Language checks / page 27

1. Which note is accurate?

D. Patient reports outside discontinuation; local list still includes medicine; instruction unavailable; referred to Dana.

The note preserves the two sources, missing instruction, and review route without inventing a treatment decision.

2. What does the active local entry establish by itself?

D. The medicine remains listed locally, not that a new instruction has been verified.

A record status does not automatically resolve a conflicting report or create a new prescription.

3. Which reply crosses Talia's stated role?

B. Restart it until we hear otherwise.

Restarting is a treatment instruction that Talia is not authorized to supply in this case.

4. Which additional conclusion is unsupported?

D. The discrepancy proves an allergy.

No reaction or allergy details are supplied; a record discrepancy cannot establish either.

03 Conversation answers

Medication histories with clear sources

E Dialogue gaps

Pages 28-29

1. patient report. Turn 2. The patient report attributes the claimed outside change to Ren without pretending the original instruction has been verified.

2. local medication list. Turn 4. The local medication list is the current organization's record, not automatic proof of a verified present treatment instruction.

3. outside instruction. Turn 6. The outside instruction is the missing source needed for appropriate verification; its absence does not prove Ren's report is false.

4. adherence. Turn 8. Adherence concerns use in relation to an agreed plan; a conflict between records does not establish deliberate nonadherence.

5. verification source. Turn 10. A verification source is the document or professional used to confirm information; Ren's attributed report is not a substitute for that review.

6. active entry. Turn 12. An active entry describes the system status and may require reconciliation; it cannot independently settle the reported outside change.

7. Strength. Turn 14. Strength describes active ingredient per product unit and is different from the amount prescribed for a dose.

8. discrepancy. Turn 16. A discrepancy is the unresolved difference between Ren's report and the local record, not a confirmed treatment error or new instruction.

9. Medication reconciliation. Turn 18. Medication reconciliation compares and resolves medication information through the authorized process; merely documenting a conflict does not complete it.

10. discontinuation. Turn 20. Discontinuation is the claimed stopping of the medicine; it remains a reported change rather than a verified decision in this conversation.

F Transfer conversation

Page 30

1. dose

Assistant: "Ellis reports an outside dose change."

The patient reports a dose change, not an established allergy.

2. unchanged

Nurse: "The local record remains unchanged."

The local record has not changed in the supplied facts.

3. unavailable

Assistant: "The outside instruction is unavailable."

The outside instruction is not available for verification here.

4. Priya

Nurse: "The receiving owner is Priya."

Priya has explicitly accepted the query for appropriate review.

04 Briefing and language checks

Laboratory paperwork and specimen queries

04

A Briefing checks / page 31

1. Which event occurred at 09:20 according to the requisition?

D. Collection

The requisition explicitly labels 09:20 as the collection time.

2. What does 10:05 describe?

D. Arrival on the courier sheet

The courier sheet records arrival, which is a different event from collection.

3. What does the interval establish by itself?

A. A 45-minute difference between the recorded events

The time difference can be calculated, but no test-specific acceptability criteria or result is supplied.

D Language checks / page 36

1. Which combined entry preserves the facts?

D. Collection 09:20; arrival 10:05

The two original times remain attached to their stated events.

2. Why should 09:20 not be replaced merely for consistency?

D. The replacement would change a collection event into an arrival time.

Matching the numbers would falsify the supplied event meaning rather than resolve a real discrepancy.

3. Which conclusion is unsupported?

D. The specimen is acceptable because only 45 minutes elapsed.

No test-specific acceptance criteria or relevant handling facts support an acceptability decision.

4. If a destination field is unclear, what is the next step?

A. Clarify the required event with the laboratory through the approved process.

The field meaning must be established before transferring a time into it.

04 Conversation answers

Laboratory paperwork and specimen queries

E Dialogue gaps

Pages 37-38

- 1. event label.** Turn 2. The event label identifies what happened at each time; different events need not have matching timestamps.
- 2. collection time.** Turn 4. The collection time is 09:20 on the requisition and must not be replaced with the separate arrival time.
- 3. arrival time.** Turn 6. The arrival time is 10:05 on the courier sheet; its meaning remains separate from collection and other laboratory events.
- 4. Receipt time.** Turn 8. Receipt time refers to the receiving service's documented event and must not be inferred from an undefined arrival field.
- 5. specimen label.** Turn 10. The specimen label supports identification; the two different event times do not themselves establish an identity mismatch.
- 6. transport interval.** Turn 12. The transport interval is a calculated difference between defined events, not an independent judgment of specimen acceptability.
- 7. Acceptance criteria.** Turn 14. Acceptance criteria are the applicable requirements for the specimen and test; they are not supplied by the time difference alone.
- 8. record amendment.** Turn 16. A record amendment is a traceable correction through the approved process, not an unexplained overwrite to make fields look consistent.
- 9. Turnaround time.** Turn 18. Turnaround time needs defined start and end events; the collection-to-arrival interval does not establish result availability.
- 10. requisition.** Turn 20. The requisition remains the source of the collection entry, while the courier sheet supplies the distinct arrival entry.

F Transfer conversation

Page 39

1. 08:40

Assistant: "Collection was recorded at 08:40."

The collection record supplies 08:40 as its event time.

2. 09:15

Colleague: "Arrival was recorded at 09:15."

The courier sheet supplies 09:15 as the arrival time.

3. distinct

Assistant: "The event labels must remain distinct."

The times refer to separate events and should not be merged.

4. clarification

Colleague: "The destination field needs clarification."

Its meaning is unclear and must be established before entry.

04

05 Briefing and language checks

Referrals and appointment dependencies

A Briefing checks / page 40

1. What happened Tuesday?

D. The specialist office received referral F8.

Tuesday is the recorded receipt event, not attendance or booking.

2. What is Friday for?

A. Lia's status check

The only Friday commitment is Lia checking the referral status.

3. What remains unconfirmed?

A. An appointment date

No appointment has been booked in the supplied facts.

D Language checks / page 45

1. Which update accurately uses all three days?

B. Sent Monday, received Tuesday, and Lia checks status Friday; no appointment is booked.

Each day is attached to its actual event, and the absence of a booking remains explicit.

2. Which sentence creates a false expectation?

D. We will see you at the specialist on Friday.

Friday is only a coordinator checkpoint, not a supplied appointment.

3. What does receipt establish?

C. The referral reached the receiving office, not that a slot is booked.

Receipt is an earlier administrative stage and does not prove later decisions or attendance.

4. How should a new clinical concern be handled?

B. Through the clinic's appropriate clinical route rather than waiting for Lia's routine check

Administrative follow-up timing does not establish clinical safety or replace assessment.

05 Conversation answers

Referrals and appointment dependencies

E Dialogue gaps

Pages 46-47

1. follow-up checkpoint. Turn 2. The follow-up checkpoint is Lia's Friday status check, not a booked time for Morgan to attend the specialist.

2. receipt acknowledgment. Turn 4. Receipt acknowledgment confirms the referral reached the specialist office; it does not establish an appointment or completed clinical review.

3. referral identifier. Turn 6. The referral identifier F8 links the conversation to the specific request, preserving which case the three dates describe.

4. Under review. Turn 8. Under review describes the current assessment stage; it is not a supplied rejection or confirmed booking.

5. referral coordinator. Turn 10. The referral coordinator owns the administrative status check here, not the specialist's clinical decision or appointment availability.

6. booking confirmation. Turn 12. Booking confirmation would establish a scheduled appointment, which is specifically absent from the supplied referral status.

7. appointment offer. Turn 14. An appointment offer is a proposed time that still needs the relevant agreement and confirmation, not automatic attendance.

8. receiving service. Turn 16. The receiving service's acknowledgment concerns referral receipt and does not by itself verify the patient's insurance coverage.

9. clinical priority. Turn 18. Clinical priority requires the appropriate clinical assessment and cannot be inferred from a routine coordinator follow-up date.

10. care coordination. Turn 20. Care coordination keeps the referral status, owner, and next action connected without falsely reporting that the specialist visit is booked.

F Transfer conversation

Page 48

1. G3

Assistant: "The referral identifier is G3."

G3 identifies the specific referral in this second scenario.

2. Wednesday

Patient: "Receipt was confirmed Wednesday."

Wednesday is the receipt day, not an appointment date.

3. under review

Assistant: "The status remains under review."

The receiving service has not supplied a booked appointment.

4. Monday

Patient: "Noor will check on Monday."

Monday is the coordinator checkpoint rather than a confirmed visit.

05

06 Briefing and language checks

Supporting questions during a clinical visit

A Briefing checks / page 49

1. Which document contains the unclear phrase?

B. The after-visit summary

The brief identifies the after-visit summary as the source.

2. Who is available to explain the clinical instruction?

C. Dr Chen

Dr Chen is available while Mina cannot reinterpret the instruction.

3. What does Jules request?

D. An explanation before leaving, away from reception

The patient requests timely clarification in an appropriate private setting.

D Language checks / page 54

1. Which response best preserves Jules's request?

A. Dr Chen is available; I will pass on this phrase and arrange the appropriate private explanation before you leave.

The response preserves the specific question, clinician route, timing, and privacy request.

2. What does handing over the summary establish?

D. The document was provided, not that the patient understands every instruction.

Document delivery and understanding are distinct events requiring different evidence.

3. Which statement misuses teach-back?

B. It is a test to prove the patient is intelligent enough.

Teach-back evaluates communication and understanding rather than judging the patient's intelligence.

4. What must remain unchanged in Mina's handoff?

C. The original instruction and the substance of the patient's question

Preserving the source and question allows the clinician to clarify without distortion.

06 Conversation answers

Supporting questions during a clinical visit

E Dialogue gaps

Pages 55-56

- 1. after-visit summary.** Turn 2. The after-visit summary is the document containing the unclear phrase, so the handoff should preserve that source.
- 2. clinical instruction.** Turn 4. A clinical instruction concerns care and requires the appropriate professional explanation; Mina cannot invent its meaning in this role.
- 3. private discussion.** Turn 6. A private discussion addresses Jules's request to ask away from reception without announcing the personal details publicly.
- 4. technical term.** Turn 8. A technical term may be familiar in appearance without being understood in its clinical context, which is why clarification remains necessary.
- 5. patient question.** Turn 10. The patient question concerns a specific phrase and its practical meaning, not merely a nonspecific request for more information.
- 6. plain language.** Turn 12. Plain language addresses understandable wording; louder repetition of the same unclear phrase does not resolve its meaning.
- 7. teach-back.** Turn 14. Teach-back helps the professional check the explanation by hearing the patient's understanding, rather than accepting a yes answer as sufficient.
- 8. instruction source.** Turn 16. The instruction source preserves the actual wording and context for Dr Chen instead of an altered summary created during the handoff.
- 9. revised plan.** Turn 18. A revised plan would require an authorized clinical change, which has not occurred merely because Jules asked for clarification.
- 10. question handoff.** Turn 20. The question handoff includes the document, specific phrase, privacy request, and need for clarification while Jules remains at the clinic.

F Transfer conversation

Page 57

1. abbreviation

Lee: "The unclear item is an abbreviation."

The supplied question concerns an abbreviation in the visit summary.

2. leaving

Ari: "I want clarification before leaving."

Ari is requesting explanation before leaving the clinic.

3. Malik

Lee: "Dr Malik is available."

Dr Malik is the available clinician identified in this scenario.

4. private

Ari: "I would like the explanation to be private."

The facts specify an appropriate private explanation with the clinician.

06

07 Briefing and language checks

Telephone concerns and clinical escalation

A Briefing checks / page 58

1. What is Ellis asking Noor to decide?

B. Whether a new concern can safely wait until next week's visit

The caller seeks a clinical timing decision rather than an administrative fact.

2. Who is available now?

A. Nurse Dana through the clinical-call route

Dana is specifically available now for the proper clinical route.

3. What does the lack of supplied symptom details establish?

D. The exercise has not established urgency or safety to wait.

Missing clinical information cannot be turned into a reassuring assessment or a negative finding.

D Language checks / page 63

1. Which response addresses the caller's need appropriately?

D. I cannot assess whether waiting is safe; Dana is available now through our clinical-call route.

The response avoids an unsupported assessment and identifies the immediate clinical route.

2. What does an unanswered transfer establish?

B. The clinical connection has not been completed.

An attempted transfer is not evidence of a received or completed clinical assessment.

3. Which information must not be invented for a structured handoff?

A. A diagnosis or urgency decision outside Noor's role

A framework organizes real information; it does not authorize fabricated clinical judgments.

4. What takes precedence when applicable?

C. The actual emergency pathway

Emergency procedures must not be delayed by a routine booking or language exercise.

07 Conversation answers

Telephone concerns and clinical escalation

E Dialogue gaps

Pages 64-65

- 1. urgency assessment.** Turn 2. Urgency assessment is a clinical judgment, not a decision Noor made when connecting the call.
- 2. triage.** Turn 4. Triage addresses urgency and care needs; asking whether a concern can wait requires more than a booking check.
- 3. routine appointment.** Turn 6. An existing routine booking does not assess a new concern or establish a safe waiting period.
- 4. clinical-call route.** Turn 8. This is the established pathway used to connect Ellis with Dana; its actual outcome must be recorded.
- 5. receiving clinician.** Turn 10. Dana confirmed receipt. Naming her does not authorize Noor to invent a clinical conclusion on her behalf.
- 6. warm transfer.** Turn 12. Noor introduced the concern and confirmed Dana received the call, rather than merely attempting an unconfirmed transfer.
- 7. dropped call.** Turn 14. A dropped call needs the actual continuity process; interruption does not establish successful clinical contact.
- 8. callback queue.** Turn 16. A callback queue holds requests awaiting contact, not evidence that a clinical conversation or assessment occurred.
- 9. emergency pathway.** Turn 18. Applicable emergency procedures take priority over administrative work or this later language-practice review.
- 10. escalation outcome.** Turn 20. Dana's receipt is confirmed; no diagnosis, completed assessment, or waiting advice is supplied here.

F Transfer conversation

Page 66

1. confirmed

Lee: "Receipt has not been confirmed."

The connection ended before anyone confirmed clinical receipt of the concern.

2. dropped-call

Colleague: "Use the clinic's dropped-call process."

The scenario specifies the established process for interrupted contact.

3. Priya

Lee: "The intended recipient is nurse Priya."

Priya is the named clinical recipient for this transfer.

4. assessment

Colleague: "No clinical assessment has occurred."

The facts explicitly state that clinical assessment has not occurred.

08 Briefing and language checks

Records, corrections, and follow-up ownership

A Briefing checks / page 67

1. What does the verified schedule show?

C. Thursday at 14:00 with Dr Chen

The schedule confirms Thursday at 14:00 with Dr Chen.

2. What caused Robin to rearrange work?

B. Elena's inaccurate Tuesday message

The case identifies the incorrect message as the reason for the work disruption.

3. Which topic is not supplied for discussion?

B. The actual test results

No result values or clinical interpretations are part of this exercise.

D Language checks / page 72

1. Which correction preserves the history?

D. My Tuesday message was wrong; the verified booking is Thursday at 14:00 with Dr Chen.

The original error was in the message, while the schedule supplies the verified Thursday booking.

2. Which conclusion about results is unsupported?

B. Thursday means the results are definitely normal.

Scheduling details do not establish the content or clinical significance of test results.

3. What should be recorded if Robin is not reached?

A. The contact attempt and the still-outstanding correction

An unsuccessful attempt does not prove that the patient received or acknowledged the corrected details.

4. Which apology is strongest?

D. I am sorry my incorrect Tuesday message disrupted your work arrangements.

The strongest apology identifies the actual message error and work disruption without shifting blame or inventing a schedule change.

08 Conversation answers

Records, corrections, and follow-up ownership

E Dialogue gaps

Pages 73-74

- 1. message correction.** Turn 2. The message correction replaces Elena's inaccurate Tuesday notification; the actual booking was not changed from Tuesday to Thursday.
- 2. schedule of record.** Turn 4. The schedule of record is the verified source showing Thursday at 14:00 with Dr Chen, rather than another tentative message.
- 3. rescheduling.** Turn 6. Rescheduling changes an actual appointment; here the booking remained Thursday and only the Tuesday message was wrong.
- 4. twenty-four-hour time.** Turn 8. Twenty-four-hour time expresses two in the afternoon as 14:00, which is the verified booking time in this case.
- 5. results discussion.** Turn 10. A results discussion identifies the purpose of the appointment and does not reveal the findings or their clinical significance.
- 6. result interpretation.** Turn 12. Result interpretation is a clinical explanation that has not occurred in this conversation, which concerns only the booking correction.
- 7. audit trail.** Turn 14. The audit trail preserves the inaccurate original message and its correction, so the sequence is not rewritten as patient error.
- 8. successful contact.** Turn 16. Successful contact means Robin actually received the corrected details; it is different from a call attempt or unsent message.
- 9. follow-up owner.** Turn 18. A follow-up owner remains responsible for unfinished communication; an unsuccessful contact attempt must not automatically close the task.
- 10. escalation route.** Turn 20. The escalation route provides a real process for the unresolved service concern without inventing an unauthorized remedy or compensation promise.

F Transfer conversation

Page 75

- 1. 09:00**
Noor: "My message incorrectly said 09:00."
The error was the 09:00 time in the message.
- 2. 11:30**
Ari: "The verified time is 11:30."
The schedule confirms 11:30 rather than the earlier message time.
- 3. Shah**
Noor: "The clinician is Dr Shah."
Dr Shah is the clinician named in the verified booking.
- 4. rescheduling**
Ari: "This is a message correction, not rescheduling."
The appointment itself did not change; only the communication was corrected.

08

09 Conversation answers

Book the right language support

E Extended conversation

Pages 76-77

- 1. spoken-language preference.** Turn 2. Mei stated Mandarin for spoken communication; Cantonese in the copied appointment note conflicts with that confirmed preference.
- 2. written-language preference.** Turn 4. Traditional Chinese is Mei's stated choice for written materials; it should not be inferred from the spoken-language field or replaced by it.
- 3. qualified interpreter.** Turn 6. The clinic has arranged a qualified healthcare interpreter for Mandarin; a relative's presence for support does not replace that booking.
- 4. video interpreting.** Turn 10. The booked service uses video for the live spoken exchange; it is not a written translation service or merely a telephone appointment reminder.
- 5. short segments.** Turn 14. Short segments let the interpreter relay each part accurately and allow the patient to ask questions without relying on a long end-of-explanation summary.
- 6. teach-back.** Turn 18. Teach-back checks the clarity of an explanation by hearing the patient's understanding in their own words; a yes response or fluent English is not the test.

F Transfer exchange

Page 78

1. Spanish

Assistant: Luis's spoken preference is Spanish .

Luis explicitly chose Spanish for conversation; the English paperwork preference does not override it.

2. English

Coordinator: His written preference is English .

The supplied record separately confirms English for written materials, rather than Spanish by assumption.

3. telephone

Assistant: The booked interpreter will join by telephone .

The booked service is telephone interpreting, not video or an already completed connection.

4. support

Coordinator: The son's attendance is for support .

The son is attending for support; his presence does not replace the arranged qualified interpreter.

Background reading: [AHRQ: language preferences and qualified language-access services](#). The conversation is an original fictional teaching case.

10 Conversation answers

A number needs its unit

E Extended conversation

Pages 79-80

1. source measurement. Turn 2. The saved scale output establishes 160.0 pounds at 09:12; appearance or the patient's previous weight would not establish this measurement's unit.

2. 72.6. Turn 4. 160.0 multiplied by 0.45359237 equals 72.5747792 kilograms, which rounds to 72.6 kilograms at one decimal place.

3. unit conversion. Turn 6. A unit conversion changes the numerical value and unit together while representing the same measured quantity; relabeling 160.0 pounds as kilograms changes its meaning.

4. height. Turn 8. The separate 170-centimeter entry describes height; its conversion to 1.70 meters is unrelated to converting the weight from pounds to kilograms.

5. audit trail. Turn 12. The audit trail preserves the original entry, correction, reason, and authorship under the clinic process; an unexplained overwrite would hide the recording error.

6. measurement time. Turn 16. The measurement occurred at 09:12; the later correction time is a separate record event and must not make the conversion look like a new weighing.

F Transfer exchange

Page 81

1. 59.9

Assistant: The weight rounds to 59.9 kilograms.

132.0 multiplied by 0.45359237 is 59.87419284, which rounds to 59.9 kilograms.

2. 10:18

Nurse: The original measurement time remains 10:18 .

The patient was weighed at 10:18; 10:31 describes the later correction, not a new weighing.

3. 1.65

Assistant: The separate height is 1.65 meters.

There are one hundred centimeters in a meter, so 165 centimeters equals 1.65 meters.

4. 10:31

Nurse: The record was corrected at 10:31 .

The supplied correction time is 10:31 and must remain distinct from the measurement time.

10

Background reading: [NIST: Handbook 44, 2026, Appendix C, mass conversion factors](#). The conversation is an original fictional teaching case.

11 Conversation answers

A clear trace is not a diagnosis

E Extended conversation

Pages 82-83

1. lead-off warning. Turn 2. A lead-off warning concerns the recording connection; it is not itself a diagnosis about the patient's heart or proof that all clinical findings are artifact.

2. electrodes. Turn 6. The ten skin electrodes acquire the signals used for a standard twelve-lead ECG; the number of electrodes is not the number of displayed electrical views.

3. Signal quality. Turn 10. Signal quality concerns the recording's clarity and interference; it is distinct from whether the patient's ECG has a clinically important finding.

4. Automated interpretation. Turn 12. Automated interpretation is the device's analysis, not a completed clinician review; its wording must not be presented as Dr Shah's confirmed conclusion.

5. repeat acquisition. Turn 16. Repeat acquisition means obtaining a new recording at 11:10; it does not erase the original 11:06 trace or establish a clinical change between them.

6. reviewing clinician. Turn 18. The reviewing clinician has received the material but must still review it; receipt alone does not establish interpretation or a discharge decision.

F Transfer exchange

Page 84

1. 10

Assistant: This standard twelve-lead recording uses 10 skin electrodes.

The standard twelve-lead ECG uses ten electrodes to obtain twelve electrical views.

2. 14:02

Clinician: The original trace is timed 14:02 .

The first recording retains 14:02 even though a later repeat was obtained.

3. 14:07

Assistant: The repeat is timed 14:07 .

The repeat is a separate acquisition at 14:07, not a correction to the first trace's timestamp.

4. supplied

Clinician: A clinical interpretation is not yet supplied .

Receipt of the traces is confirmed, but the facts do not supply a clinical interpretation or clearance.

11

Background reading: [Royal Papworth Hospital: standard ECG electrodes and electrical leads](#). The conversation is an original fictional teaching case.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Clarifying the visit at check-in

Use the preferred name: Thank you, Alex; I will use that name.

Name the difference: The recorded visit type and your expectation are not the same.

Avoid blame: Let us clarify the plan before assuming how the mismatch happened.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Rooming and factual patient reports

Attribute the words: Pat reports feeling tired since Monday.

Name the limit: I have not assessed the cause or severity.

Avoid a vague reassurance: I cannot describe the concern as mild from that observation.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Medication histories with clear sources

Acknowledge the report: You report that another clinic stopped this medicine.

Avoid judging memory: I will record what you remember and ask the clinical team to check the difference.

Avoid treatment advice: I cannot tell you to start, stop, or restart it.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Laboratory paperwork and specimen queries

Identify the first field: The requisition says collection at 09:20.

Question the proposed change: Why would an arrival time replace a collection time?

Clarify an ambiguous field: Let us ask the laboratory which event this field requires.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Referrals and appointment dependencies

Identify the referral: We are discussing referral F8.

Explain the stage: They report that it is under review.

Avoid a review guarantee: We do not have a confirmed review-completion date.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Supporting questions during a clinical visit

Welcome the question: Thank you for telling me which part is unclear.

State the boundary: I cannot reinterpret this clinical instruction.

Avoid a guessed meaning: I do not want to give you an explanation that changes the instruction.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Telephone concerns and clinical escalation

Acknowledge the question: You want to know whether this can wait until next week.

Avoid unsupported reassurance: I do not have a clinical assessment that would support that reassurance.

Confirm the recipient: I will identify the person receiving the concern.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Records, corrections, and follow-up ownership

Open the correction: I need to correct the appointment message I sent you.

Translate the time: Fourteen hundred is two in the afternoon.

Keep the role clear: Dr Chen will handle the clinical discussion.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

abbreviation expand an abbreviation	50
acceptance criteria check acceptance criteria	33
access barrier identify an access barrier	42
accessible format provide an accessible format	51
accession number confirm the accession number	32
acknowledgment record acknowledgment	69
active entry verify an active entry	23
addendum enter an addendum	69
adherence discuss adherence without blame	24
adverse reaction report an adverse reaction	24
after-visit summary review the after-visit summary	50
allergy history document allergy history	24
ambiguity resolve an ambiguity	50
analytical phase distinguish the analytical phase	33
annual review attend an annual review	5
appointment availability check appointment availability	41
appointment confirmation send an appointment confirmation	68
appointment offer confirm an appointment offer	42
appointment slot confirm the appointment slot	6
appointment type verify the appointment type	5
arrival time verify the arrival time	32
as-needed instruction verify the as-needed instruction	24
associated symptom ask about associated symptoms	14
attendance confirm attendance	42

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

audit trail maintain an audit trail	69
baseline establish the baseline	14
billing category verify the billing category	6
blind transfer avoid an unsupported blind transfer	59
blood pressure document blood pressure	15
booking confirmation obtain booking confirmation	41
call disposition document the call disposition	59
callback commitment confirm a callback commitment	59
callback queue check the callback queue	59
cancellation slot check a cancellation slot	42
care coordination support care coordination	42
change from baseline report a change from baseline	14
check-in complete check-in	5
chief complaint clarify the chief complaint	5
chunk and check use chunk and check	50
clarification request clarification	50
clinical assessment request clinical assessment	15
clinical escalation initiate clinical escalation	59
clinical handoff complete a clinical handoff	6
clinical instruction clarify a clinical instruction	50
clinical priority refer clinical-priority questions	42
clinical suitability refer a suitability question	6
clinical-call route use the clinical-call route	59
closed-loop communication use closed-loop communication	60

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

closure evidence	retain closure evidence	69
collection method	document the collection method	32
collection time	record the collection time	32
communication log	update the communication log	69
consultation report	obtain the consultation report	42
contact attempt	document a contact attempt	69
correction timestamp	preserve the correction timestamp	68
courier sheet	reconcile the courier sheet	32
date verification	complete date verification	68
delegation	clarify the delegation	6
demographic details	verify demographic details	5
dependency	identify a booking dependency	42
dietary supplement	ask about dietary supplements	24
discontinuation	confirm discontinuation	23
discrepancy	flag a medication discrepancy	23
dosage form	confirm the dosage form	24
dose	verify the prescribed dose	24
dropped call	follow the dropped-call process	60
duplicate entry	investigate a duplicate entry	23
duration	document symptom duration	14
emergency pathway	follow the emergency pathway	60
encounter	document the encounter	5
escalation outcome	record the escalation outcome	60
escalation route	explain the escalation route	69

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

event label preserve the event label	33
follow-up checkpoint set a follow-up checkpoint	41
follow-up owner name the follow-up owner	69
frequency clarify the frequency	24
functional impact describe functional impact	14
health literacy support health literacy	51
hemolysis report hemolysis	33
historical entry distinguish a historical entry	23
identity check complete the identity check	60
identity match establish the identity match	6
incorrect notification correct an incorrect notification	68
instruction source identify the instruction source	51
insufficient quantity report insufficient quantity	33
interpretation clarify the required interpretation	51
jargon clarify unfamiliar jargon	50
language access arrange language access	51
local medication list review the local medication list	23
measurement time preserve the measurement time	15
medication history obtain a medication history	23
medication reconciliation support medication reconciliation	23
message correction issue a message correction	68
name pronunciation check name pronunciation	6
new concern report a new concern	59
objective finding document an objective finding	14

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

observation	record an observation	14
omission	identify a possible omission	23
onset	clarify the reported onset	14
original entry	preserve the original entry	69
outside instruction	verify the outside instruction	23
outstanding action	track the outstanding action	69
over-the-counter medicine	record over-the-counter medicines	24
oxygen saturation	report oxygen saturation	15
patient expectation	acknowledge the patient expectation	6
patient identifier	confirm patient identifiers	5
patient question	preserve the patient question	51
patient report	document the patient report	23
patient-reported symptom	attribute a patient-reported symptom	14
pertinent negative	document a pertinent negative	15
plain language	explain in plain language	50
postanalytical phase	describe the postanalytical phase	33
preanalytical phase	review the preanalytical phase	32
preferred name	use the preferred name	5
prescribing clinician	contact the prescribing clinician	23
presenting concern	record the presenting concern	5
private discussion	arrange a private discussion	51
pulse rate	measure pulse rate	15
qualified interpreter	request a qualified interpreter	51
question handoff	complete a question handoff	51

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

read-back request a read-back	68
reassurance avoid unsupported reassurance	60
receipt acknowledgment obtain a receipt acknowledgment	41
receipt time document the receipt time	32
receiving clinician identify the receiving clinician	59
receiving service contact the receiving service	41
record amendment make a record amendment	33
recorded name verify the recorded name	5
red flag escalate a reported red flag	60
referral send a referral	41
referral completion document referral completion	42
referral coordinator name the referral coordinator	41
referral criteria clarify referral criteria	42
referral identifier quote the referral identifier	41
referring clinician identify the referring clinician	41
registration complete registration	5
reinterpretation avoid unauthorized reinterpretation	50
rejection criteria apply rejection criteria	33
requisition check the requisition	32
rescheduling distinguish rescheduling from correction	68
respiratory rate record respiratory rate	15
result interpretation refer result interpretation	68
results discussion confirm a results discussion	68
revised plan confirm a revised plan	51

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

rooming complete the rooming process	14
route verify the route	24
routine appointment distinguish a routine appointment	59
safety-net advice provide authorized safety-net advice	60
SBAR use SBAR within the actual role	60
schedule of record check the schedule of record	68
scheduled purpose state the scheduled purpose	6
scope of practice work within scope of practice	6
sent status verify the sent status	41
service disruption acknowledge the service disruption	69
severity assess severity	14
show-me method use the show-me method	50
source attribution preserve source attribution	15
specialist office check with the specialist office	41
specimen identify the specimen	32
specimen integrity assess specimen integrity	33
specimen label verify the specimen label	32
specimen source record the specimen source	32
strength distinguish strength from dose	24
subjective information attribute subjective information	14
successful contact verify successful contact	69
supervision confirm supervision arrangements	6
supporting documentation supply supporting documentation	42
symptom onset relay symptom onset	60

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

symptom progression	describe symptom progression	60
teach-back	use teach-back appropriately	50
technical term	explain a technical term	50
temperature	record the measured temperature	15
temperature condition	document temperature conditions	33
time-zone reference	clarify the time-zone reference	68
translation	obtain an appropriate translation	51
transport interval	calculate the transport interval	32
transport medium	verify the transport medium	33
triage	refer for triage	59
turnaround time	define the turnaround time	33
twenty-four-hour time	use twenty-four-hour time	68
unassessed	mark severity as unassessed	15
under review	explain under review	41
understanding check	conduct an understanding check	51
unit of measurement	include the unit of measurement	15
urgency assessment	obtain an urgency assessment	59
verification source	identify the verification source	24
verified callback number	obtain a verified callback number	60
visit agenda	clarify the visit agenda	5
visit plan	clarify the visit plan	6
vital signs	record measured vital signs	15
waiting list	verify waiting-list status	42
warm transfer	arrange a warm transfer	59

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] American Association of Medical Assistants. State Scope of Practice Laws.

[Open the original source](#)

US context for jurisdiction-dependent duties and delegation. The fictional roles in this book do not authorize clinical work. Checked 10 October 2026.

[2] AHRQ. Use the Teach-Back Method: Tool 5.

[Open the original source](#)

Communication context for checking an explanation without shaming the patient. Clinical instruction questions remain with the appropriately authorized professional. Checked 10 October 2026.

[3] AHRQ. Make Referrals Easy: Tool 21.

[Open the original source](#)

Background on referral coordination, clear instructions, and follow-up. Original referral cases separate receipt, review, booking, and completed attendance. Checked 10 October 2026.

[4] CDC. Collect Adult Blood Culture Sets.

[Open the original source](#)

A specific laboratory example distinguishing collection documentation and specimen receipt. The book does not teach blood collection or universal specimen handling. Checked 10 October 2026.

Phil Stilwell

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